



SHC Virtual Location
5651 Copley Drive; Suite A
SAN DIEGO CA 92111-7903

Peikert, Thomas
MRN: 200306042, DOB: 3/9/1988, Legal Sex: M
Adm: 6/29/2024, D/C: 6/29/2024

06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room

ED Provider Note

ED Provider Notes by James Hsiao, MD at 6/29/2024 0932

Author: James Hsiao, MD
Filed: 6/29/2024 2:30 PM
Editor: James Hsiao, MD (Physician)

Service: Emergency Medicine
Date of Service: 6/29/2024 9:32 AM

Author Type: Physician
Status: Signed

ED NOTE

Date of Service: 6/29/2024
Mode of Arrival: Ambulance
PCP: Nathan A Mcfarland, MD
Provider: James Hsiao, MD

CHIEF COMPLAINT

Chief Complaint

Patient presents with

- Back Pain

HISTORY OF PRESENT ILLNESS

This is a 36-year-old male who was brought in by medics for head pain, neck pain, and back pain being after involved in a motor vehicle collision, which occurred shortly prior to arrival. The patient was the restrained driver of a car traveling at approximately 35 mph when his car was T-boned by another vehicle on the passenger side. The airbags on the right side were deployed, but not on his own side. He is not sure if he hit his head with the MVC, and thinks he may have lost consciousness, but only for a brief second. However, he was able to self extricate himself from the car and has been ambulatory since the event. He now complains of occipital head pain, radiating down the posterior neck. He also complains of generalized back pain, involving both sides of the upper and lower back, but denies any weakness or paresthesias in the extremities. He denies any anterior chest pain, abdominal pain, or shortness of breath. He reports feeling slightly nauseous, but denies any vomiting. He does not take any anticoagulants.

PAST MEDICAL HISTORY

No past medical history on file.

PAST SURGICAL HISTORY

No past surgical history on file.

MEDICATIONS

Current Outpatient Medications

Medication	Instructions
• ibuprofen	600 mg, oral, Every 6 hours PRN
• ondansetron ODT (ZOFTRAN-ODT)	4 mg, oral, Every 8 hours PRN

The patient's available Medication List was reviewed and updated to the extent possible during today's evaluation.

ALLERGIES

Patient has no known allergies.



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

ED Provider Note (continued)

SOCIAL HISTORY

Social History

Social History Narrative

- Not on file

Lives in SPRING VALLEY CA 91977-4*

The patient denies current use of tobacco products.

FAMILY HISTORY

No family history of early cardiac disease.

REVIEW OF SYSTEMS

A 10-point review of systems was obtained and found to be negative except for those mentioned in the HPI.

PHYSICAL EXAM

Physical Exam

ED Triage Vitals

Temp	Heart Rate	Resp	BP
06/29/24	06/29/24	06/29/24	06/29/24
0836	0836	0836	0836
37.2 °C (99 °F)	100	20	150/88

SpO2	Temp	Heart Rate	Patient
	Source	Source	Position
06/29/24	06/29/24	06/29/24	06/29/24
0836	0836	1100	1100
98 %	Oral	Monitor	Sitting

BP Location FiO2 (%)

06/29/24 --

0836

Left arm

Pulse oximetry was interpreted by me as being 98% on room air, which is within normal limits.

CONSTITUTIONAL: The patient is in no apparent distress. The patient is non-toxic in appearance.

SKIN: No rashes or lesions noted to the exposed portions of the skin.

HEAD: Normocephalic, atraumatic. Pupils are equally round and reactive to light and accommodation. Extraocular movements are intact.

NECK: Neck is supple. No meningismus. No posterior midline tenderness along the cervical spine. There is no neck swelling or hematoma, and no external signs of trauma to the neck.



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

ED Provider Note (continued)

PULMONARY: Lungs are clear to auscultation bilaterally, without any crackles or wheezing.

CARDIAC: Regular rhythm and rate.

ABDOMEN: Soft, non-tender, and non-distended. Normoactive bowel sounds are present.

GU: No costovertebral angle tenderness.

BACK: No focal midline tenderness along the thoracic or lumbosacral spine. No external signs of trauma to the back.

EXTREMITIES: Lower extremities are symmetric in size and appearance.

NEUROLOGIC: The patient has no pronator drift. The patient has symmetric strength and intact sensation to gross touch in the bilateral upper and lower extremities. Cranial nerves II through XII are grossly intact. The patient has clear and comprehensible speech, without any evidence of dysarthria or aphasia.

DIAGNOSTIC DATA

LABORATORY STUDIES

The following tests were ordered and reviewed by me during this encounter:

Labs Reviewed - No data to display

RADIOLOGY STUDIES

The following radiology studies performed today were ordered by me and where indicated also interpreted by me:

----- INTERPRETATIONS BY RADIOLOGIST: -----

CT Head WO IV Contrast

Final Result

IMPRESSION:

1. No evidence for acute intracranial abnormality, midline shift or mass effect.

Study CTDI vol Max: 38.67 mGy; Study DLP Sum:

1144.38 mGy*cm; Associated accessions:

11240629002032

Finalized 6/29/2024 10:29 AM by Scott Honowitz

CT Cervical Spine WO IV Contrast

Final Result

IMPRESSION:

1. No acute osseous abnormality in the cervical spine.

Study CTDI vol Max: 38.67 mGy; Study DLP Sum:

1144.38 mGy*cm; Associated accessions:

11240629002023,11240629002032

Finalized 6/29/2024 10:32 AM by Scott Honowitz



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

ED Provider Note (continued)

XR Thoracic Spine 3 Views

Final Result

IMPRESSION:

* Normal exam.

Finalized 6/29/2024 10:13 AM by Zachary Alexander

XR Lumbar Spine 2-3 Views

Final Result

IMPRESSION:

No acute bony abnormality.

Finalized 6/29/2024 10:13 AM by Scott Honowitz

MEDICATIONS ADMINISTERED

Medications - No data to display

Procedures

ED Course & MDM

Diagnoses as of 06/29/24 1430

Injury of head, initial encounter

Neck sprain, initial encounter

Back strain, initial encounter

No orders to display

Orders Placed This Encounter

Procedures

- CT Head WO IV Contrast
- CT Cervical Spine WO IV Contrast
- XR Thoracic Spine 3 Views
- XR Lumbar Spine 2-3 Views

Medical Decision Making

This is a 36-year-old male who presents with head pain, neck pain, and generalized back pain after being involved in



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)**ED Provider Note (continued)**

an MVC. He underwent CAT scans of the head and cervical spine, both of which demonstrated no acute traumatic injury. He underwent x-rays of the thoracic and lumbar spine, both of which demonstrated no acute fracture or malalignment. The patient likely strained his back with the MVC, and I see no indication for any further advanced CT imaging at this time, but the patient has been given strict return precautions. The patient has remained awake and appropriately responsive, with no focal neurologic deficits on clinical exam. The patient was provided with copies of the imaging reports, and will now be discharged home.

Disposition: Discharge

Diagnosis:

Final diagnoses:

[S09.90XA] Injury of head, initial encounter

[S13.9XXA] Neck sprain, initial encounter

[S39.012A] Back strain, initial encounter

DISCHARGE MEDICATIONS:

Discharge Medication List as of 6/29/2024 11:08 AM

START taking these medications

	Details
ibuprofen 600 mg tablet	Take 1 tablet (600 mg) by mouth every 6 hours if needed for mild 1-4 pain for up to 7 days., Starting Sat 6/29/2024, Until Sat 7/6/2024 at 2359, Normal
ondansetron ODT (Zofran-ODT) 4 mg disintegrating tablet	Take 1 tablet (4 mg) by mouth every 8 hours if needed for nausea or vomiting for up to 7 days., Starting Sat 6/29/2024, Until Sat 7/6/2024 at 2359, Normal

FOLLOW UP

Nathan A McFarland, MD
10039 VINE ST
Lakeside CA 92040
619-390-9975

In 2 days

Return for worsening pain symptoms, weakness in extremities, difficulty ambulating, frequent falls, intractable



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)**ED Provider Note (continued)**

vomiting, or for any other new symptoms or concerns.

Electronically Signed by James Hsiao, MD

James Hsiao, MD
06/29/24 1430

Electronically signed by James Hsiao, MD at 6/29/2024 2:30 PM

ED Notes**ED Triage Notes by Tawny Thongsory at 6/29/2024 0837**

Author: Tawny Thongsory

Service: —

Author Type: Registered Nurse

Filed: 6/29/2024 8:38 AM

Date of Service: 6/29/2024 8:37 AM

Status: Signed

Editor: Tawny Thongsory (Registered Nurse)

PT BIBM from nearby street, per medic pt was travelling approx 35 mphs when he was tboned by another car. + airbags to right side of vehicle/passenger side of impact. Denies loc/thinners. PT had seatbelt on, able to self extricate. States 8/10 to generalized back. No noted obvious deformity noted. Denies head/neck pain.

Electronically signed by Tawny Thongsory at 6/29/2024 8:38 AM

Clinical Notes**Discharge Instructions****James Hsiao, MD at 6/29/2024 1045**

Author: James Hsiao, MD

Service: —

Author Type: Physician

Filed: 6/29/2024 10:45 AM

Date of Service: 6/29/2024 10:45 AM

Status: Written

Editor: James Hsiao, MD (Physician)

Electronically signed by James Hsiao, MD at 6/29/2024 10:45 AM

Imaging**Imaging****XR Thoracic Spine 3 Views (Final result)**

Status: **Completed**

Electronically signed by: **James Hsiao, MD on 06/29/24 0927**

This order may be acted on in another encounter.

Ordering user: James Hsiao, MD 06/29/24 0927

Ordering provider: James Hsiao, MD

Authorized by: James Hsiao, MD

Ordering mode: Standard

Frequency: STAT Once 06/29/24 0927 - 1 occurrence

Class: Hospital Performed

Quantity: 1

Lab status: Final result

Indications of use: Upper back injury, acute

Instance released by: James Hsiao, MD (auto-released)
6/29/2024 9:27 AM

Questionnaire**Question****Answer**



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

Imaging (continued)

Release to patient

Immediate

XR Thoracic Spine 3 Views

Resulted: 06/29/24 1013, Result status: Final result

Ordering provider: James Hsiao, MD 06/29/24 0927

Order status: Completed

Resulted by: Zachary Alexander, MD

Filed by: Interface, Radiology Results In 06/29/24 1018

Performed: 06/29/24 0954 - 06/29/24 1014

Accession number: 11240629001919

Resulting lab: POWERSCRIBE

Narrative:

HISTORY: Trauma with back pain

COMPARISON: None

TECHNIQUE: 3 views of the thoracic spine were obtained.

FINDINGS:

Bones: The vertebral bodies heights are normal, and no alignment abnormalities are seen.

Disk Spaces: The intervertebral disc heights are preserved.

Soft Tissues: Soft tissues are unremarkable.

Impression:

IMPRESSION:

* Normal exam.

Finalized 6/29/2024 10:13 AM by Zachary Alexander

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
258 - PS360	POWERSCRIBE	Unknown	Unknown	12/28/22 1429 - Present

XR Thoracic Spine 3 Views

Resulted: 06/29/24 0954, Result status: In process

Ordering provider: James Hsiao, MD 06/29/24 0927

Order status: Completed

Resulted by: Zachary Alexander, MD

Filed by: Vanessa Revilla 06/29/24 0954

Performed: 06/29/24 0954 - 06/29/24 1014

Accession number: 11240629001919

Resulting lab: IMAGING

Signed

Electronically signed by Zachary Alexander, MD on 6/29/24 at 1013 PDT

XR Lumbar Spine 2-3 Views (Final result)

Status: Completed

Electronically signed by: James Hsiao, MD on 06/29/24 0927

This order may be acted on in another encounter.

Ordering user: James Hsiao, MD 06/29/24 0927

Ordering provider: James Hsiao, MD

Authorized by: James Hsiao, MD

Ordering mode: Standard

Frequency: STAT Once 06/29/24 0927 - 1 occurrence

Class: Hospital Performed

Quantity: 1

Lab status: Final result

Indications of use: Lower back pain, acute

Instance released by: James Hsiao, MD (auto-released)
6/29/2024 9:27 AM

Questionnaire

Question	Answer
Release to patient	Immediate



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

Imaging (continued)

XR Lumbar Spine 2-3 Views

Resulted: 06/29/24 1013, Result status: Final result

Ordering provider: James Hsiao, MD 06/29/24 0927

Order status: Completed

Resulted by: Scott Honowitz, MD

Filed by: Interface, Radiology Results In 06/29/24 1017

Performed: 06/29/24 0954 - 06/29/24 1014

Accession number: 11240629001920

Resulting lab: POWERSCRIBE

Narrative:

HISTORY: Acute lower back pain

COMPARISON: None

TECHNIQUE: 3 views of the lumbar spine were obtained.

FINDINGS: Diffuse osteopenia. Normal spinal alignment aside from minimal levocurvature. Vertebral body heights are preserved, aside from multilevel Schmorl's nodes. Disc heights are preserved. No significant facet arthropathy. No definite acute displaced fracture.

Impression:

IMPRESSION:

No acute bony abnormality.

Finalized 6/29/2024 10:13 AM by Scott Honowitz

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
258 - PS360	POWERSCRIBE	Unknown	Unknown	12/28/22 1429 - Present

XR Lumbar Spine 2-3 Views

Resulted: 06/29/24 0954, Result status: In process

Ordering provider: James Hsiao, MD 06/29/24 0927

Order status: Completed

Resulted by: Scott Honowitz, MD

Filed by: Vanessa Revilla 06/29/24 0954

Performed: 06/29/24 0954 - 06/29/24 1014

Accession number: 11240629001920

Resulting lab: IMAGING

Signed

Electronically signed by Scott Honowitz, MD on 6/29/24 at 1013 PDT

CT Head WO IV Contrast (Final result)

Status: Completed

Electronically signed by: James Hsiao, MD on 06/29/24 0926

This order may be acted on in another encounter.

Ordering user: James Hsiao, MD 06/29/24 0926

Ordering provider: James Hsiao, MD

Authorized by: James Hsiao, MD

Ordering mode: Standard

Frequency: STAT Once 06/29/24 0927 - 1 occurrence

Class: Hospital Performed

Quantity: 1

Lab status: Final result

Indications of use: Injury, head, initial encounter

Instance released by: James Hsiao, MD (auto-released)

6/29/2024 9:26 AM

Questionnaire

Question	Answer
What is the patient's sedation requirement?	No Sedation
Release to patient	Immediate

CT Head WO IV Contrast

Resulted: 06/29/24 1029, Result status: Final result

Ordering provider: James Hsiao, MD 06/29/24 0926

Order status: Completed

Resulted by: Scott Honowitz, MD

Filed by: Interface, Radiology Results In 06/29/24 1033



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06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)**Imaging (continued)**

Performed: 06/29/24 1011 - 06/29/24 1023

Accession number: 11240629002032

Resulting lab: POWERSCRIBE

Narrative:

HISTORY: Injury, head, initial encounter

TECHNIQUE: Multidetector noncontrast CT scan of the head was performed from the skull base to the vertex. Axial images were obtained and both coronal and sagittal images reconstructed. Radiation dose limiting techniques were utilized including automated exposure control, adjustment of mA and/or kV to patient size according to ALARA (As Low As Reasonably Achievable) principles, and use of iterative reconstruction data processing techniques.

COMPARISON: None

FINDINGS: There is no hydrocephalus, midline shift, mass effect or intracranial hemorrhage.

There is no abnormal density in the brain parenchyma, and the gray-white differentiation is maintained.

The calvarium is intact. The mastoid air cells are clear.

The paranasal sinuses are clear.

The globes and orbits are intact.

Impression:

IMPRESSION:

1. No evidence for acute intracranial abnormality, midline shift or mass effect.

Study CTDI vol Max: 38.67 mGy; Study DLP Sum: 1144.38 mGy*cm; Associated accessions: 11240629002032

Finalized 6/29/2024 10:29 AM by Scott Honowitz

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
258 - PS360	POWERSCRIBE	Unknown	Unknown	12/28/22 1429 - Present

CT Head WO IV Contrast

Resulted: 06/29/24 1011, Result status: In process

Ordering provider: James Hsiao, MD 06/29/24 0926

Order status: Completed

Resulted by: Scott Honowitz, MD

Filed by: Steven Cuff 06/29/24 1011

Performed: 06/29/24 1011 - 06/29/24 1023

Accession number: 11240629002032

Resulting lab: IMAGING

Signed

Electronically signed by Scott Honowitz, MD on 6/29/24 at 1029 PDT

CT Cervical Spine WO IV Contrast (Final result)

Status: **Completed**

Electronically signed by: **James Hsiao, MD on 06/29/24 0926**

This order may be acted on in another encounter.

Ordering user: James Hsiao, MD 06/29/24 0926

Ordering provider: James Hsiao, MD

Authorized by: James Hsiao, MD

Ordering mode: Standard

Frequency: STAT Once 06/29/24 0927 - 1 occurrence

Class: Hospital Performed

Quantity: 1

Lab status: Final result

Indications of use: Neck injury, acute

Instance released by: James Hsiao, MD (auto-released)



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Adm: 6/29/2024, D/C: 6/29/2024

06/29/2024 - ED in Sharp Grossmont Hospital Emergency Room (continued)

Imaging (continued)

6/29/2024 9:26 AM

Questionnaire

Question	Answer
Release to patient	Immediate
What is the patient's sedation requirement?	No Sedation
Use of contrast material at the discretion of the radiologist	Yes

CT Cervical Spine WO IV Contrast

Resulted: 06/29/24 1032, Result status: Final result

Ordering provider: James Hsiao, MD 06/29/24 0926
Resulted by: Scott Honowitz, MD
Performed: 06/29/24 1011 - 06/29/24 1023
Resulting lab: POWERSCRIBE
Narrative:
HISTORY: Neck injury, acute

Order status: Completed
Filed by: Interface, Radiology Results In 06/29/24 1036
Accession number: 11240629002023

COMPARISON: None

TECHNIQUE: Axial images were obtained through the cervical spine. Sagittal and coronal reformatted images were then constructed. Radiation dose limiting techniques were utilized including automated exposure control, adjustment of mA and/or kV to patient size according to ALARA (As Low As Reasonably Achievable) principles, and use of iterative reconstruction data processing techniques.

FINDINGS: There is normal cervical vertebral body height. Minimal reversal of the cervical lordosis, nonspecific. No acute fracture is identified. The precervical soft tissues are within normal limits. Paraspinal soft tissues are unremarkable. Visualized lung apices are clear.

Impression:

IMPRESSION:

1. No acute osseous abnormality in the cervical spine.

Study CTDI vol Max: 38.67 mGy; Study DLP Sum: 1144.38 mGy*cm; Associated accessions:
11240629002023, 11240629002032

Finalized 6/29/2024 10:32 AM by Scott Honowitz

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
258 - PS360	POWERSCRIBE	Unknown	Unknown	12/28/22 1429 - Present

CT Cervical Spine WO IV Contrast

Resulted: 06/29/24 1011, Result status: In process

Ordering provider: James Hsiao, MD 06/29/24 0926
Resulted by: Scott Honowitz, MD
Performed: 06/29/24 1011 - 06/29/24 1023
Resulting lab: IMAGING

Order status: Completed
Filed by: Steven Cuff 06/29/24 1011
Accession number: 11240629002023

Signed

Electronically signed by: Scott Honowitz, MD on 6/29/24 at 1032 PDT

Chart

Mr. Thomas Peikert (2)

Patient Number: 1002

Date of Birth: 1988-03-09

852 paraíso ave

Spring valley, CA, 91977

Tel: +16195810650

Email: peikertt@gmail.com

November 16, 2024 Added by: Zach Beatty - EIN #862284215, License #DC33549, NPI #1356098412

Chief Complaint

The patient reports being struck from the side on 11/13/2024. He notes that immediately after the impact, he began experiencing upper mid back and neck tension, along with a headache. He states that upon waking up the next day, he developed tightness and pain in his low back, mid back, and neck, with worsening headaches. He also reports pain in both shoulders and tingling and pain radiating down both arms, hands, and legs. The patient mentions a prior motor vehicle accident (MVA) a few months ago, for which he sought care and had been feeling better. However, he states that the recent collision has significantly aggravated his symptoms and introduced new pain in both shoulders. He reports that his pain has progressively worsened, significantly impacting his ability to perform daily activities, sleep, work, and spend time with his family. Additionally, he mentions experiencing dizziness, forgetfulness, increased irritability, decreased appetite, and fatigue since the impact. Patient presents today with unresolved pains in the aforementioned areas. Chaperone is Sergio in the room.

Vitals

Temperature	Blood Pressure	Weight	Height
98.4 °F	122/77 mmHG	240 lbs	511 in

Cervical Range Of Motion**Right Rotation (RR)**

☐ 0° ☐ 5° ☒ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45° ☐ 50° ☐ 55° ☐ 60° ☐ 65° ☐ 70° ☐ 75° ☐ 80°

Left Rotation (LR)

☐ 0° ☐ 5° ☐ 10° ☒ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45° ☐ 50° ☐ 55° ☐ 60° ☐ 65° ☐ 70° ☐ 75° ☐ 80°

Flexion (FLEX)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45° ☐ 50°

Extension (EXT)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45° ☐ 50° ☐ 55° ☐ 60° ☐ 65° ☐ 70° ☐ 75° ☐ 80°

Right Lateral Flexion (RLF)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45°

Left Lateral Flexion (LLF)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45°

Smart Options & Narrative

Right Rotation: Tight and Painful

Left Rotation: Tight and Painful

Flexion: Tight and Painful

Extension: Tight and Painful

Right Lateral Flexion: Tight and Painful

Left Lateral Flexion: Tight and Painful

Thoracic Range Of Motion

Right Rotation (RR)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30°

Left Rotation (LR)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30°

Smart Options & Narrative

Right Rotation: Tight and Painful

Left Rotation: Tight and Painful

Lumbar Range Of Motion

Extension (EXT)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25° ☐ 30° ☐ 35° ☐ 40° ☐ 45° ☐ 50° ☐ 55° ☐ 60° ☐ 65° ☐ 70° ☐ 75° ☐ 80°

Right Lateral Flexion (RLF)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25°

Left Lateral Flexion (LLF)

☐ 0° ☒ 5° ☐ 10° ☐ 15° ☐ 20° ☐ 25°

Smart Options & Narrative

Extension: Tight and Painful

Right Lateral Flexion: Tight and Painful

Left Lateral Flexion: Tight and Painful

Smart Options & Narrative

Jacksons: Positive Right and Positive Left

Foraminal Compression: N/A

Smart Options & Narrative

Kemp's: Positive Right and Positive Left

OTHER NOTES:

Tenderness and Restrictions

ROM measured with Goniometer.

Zach Beatty

EIN #862284215, License #DC33549, NPI #1356098412

Signed on Monday August 11, 2025 at 6:00pm

November 16, 2024 Added by: Zach Beatty - EIN #862284215, License #DC33549, NPI #1356098412

Subjective

Chief Complaint

The patient reports being struck from the side on 11/13/2024. He notes that immediately after the impact, he began experiencing upper mid back and neck tension, along with a headache. He states that upon waking up the next day, he developed tightness and pain in his low back, mid back, and neck, with worsening headaches. He also reports pain in both shoulders and tingling and pain radiating down both arms, hands, and legs. The patient mentions a prior motor vehicle accident (MVA) a few months ago, for which he sought care and had been feeling better. However, he states that the recent collision has significantly aggravated his symptoms and introduced new pain in both shoulders. He reports that his pain has progressively worsened, significantly impacting his ability to perform daily activities, sleep, work, and spend time with his

family. Additionally, he mentions experiencing dizziness, forgetfulness, increased irritability, decreased appetite, and fatigue since the impact. Patient presents today with unresolved pains in the aforementioned areas. Chaperone is Sergio in the room.

Objective

Subluxation Listings

Adjusted: low back, thoracic, neck -activator

Palpatory Findings

Patient is:: Tender, Tight/tense, and Guarding/Spasm

Assessment

Current Status

Patient is currently:: Worsening

Progress

Patient is Progressing:: Slow But Steady

Plan

Care

Type of Care:: Acute/Relief Care

Proposed In-Office Treatments

Chiropractic Adjustments, Manual Therapy Technique, Therapeutic Procedures/Exercises.

Frequency & Duration

☑ Acute Care: 2x/week for 4-6 weeks

Goals of Treatment

SHORT-TERM: Decrease swelling and inflammation to the affected regions, improve core strength and stability, improve patient's tolerance for the activities of daily living, reduce muscle pain and spasm, increase range of motion.

LONG-TERM: Attain pre-condition/pre-injury status (if applicable), restore functional independence and tolerance for normal activities of daily living, restore strength and stability to affected regions.

Home/Self Care Recommendations

I recommend the patient:: Alternate ice/heat and Avoid activities and postures that aggravate condition

Recommendations

Referring patient for:: Additional diagnostic imaging and Neuro Consult

Treatment

Procedure Rationale

98940, 98941, 98943: Promote proper alignment and function; Correct subluxation.

97140: Increase ROM; Improve ADLs, Improve function.

97110: Improve strength and conditioning; Prevent muscle atrophy.

Myofascial of levator scaps & QL: 10 minutes

Patient responded favorably to today's visit.

Recommend patient get cervical and lumbar MRI due to point tenderness and radicular symptoms following trauma.

Recommend patient see a specialist due to concussion like symptoms.

Recommend patient see a specialist for shoulder if pains do not resolve as expected.

Zach Beatty

EIN #862284215, License #DC33549, NPI #1356098412

Signed on Monday August 11, 2025 at 6:00pm

PAUL A. JAIN, M.D., Q.M.E.

3838 Camino Del Rio N, STE 240
San Diego, CA 92108
(619) 880-8844

Other locations: San Marcos, Chula Vista, Murrieta, La Mesa, Santee

*Diplomate, American Board of
Internal Medicine*

*Diplomate, National Board of
Physicians and Surgeons*

Qualified Medical Examiner

December 12, 2024

RE: Piekert, Thomas
D/B: 03/09/1988
D/I: 11/13/2024
D/E: 12/12/2024

INITIAL INTERNAL MEDICINE REPORT

Mr. Thomas Piekert is a 36 year old male who was seen in my Mission Valley office on 12/12/2024 for an initial Internal Medicine evaluation in reference to the injuries he sustained as a result of a motor vehicle collision that occurred on 11/13/2024.

The patient's current complaints include neck, mid/low back and bilateral shoulder pain associated with intermittent numbness in the upper extremities. In addition, he reports persistent mild/moderate headaches, dizziness, blurry vision, brain fog, and decreased memory, attention and concentration, anxiety and insomnia since his MVC.

HISTORY OF PRESENT ILLNESS

Mr. Piekert reports that on 11/13/2024, he was the restrained passenger of a Kia Forte which was side-swiped on the drivers side by a Nissan SUV that was quickly merging into their lane. The accident occurred while they were on a freeway. He estimates that the other vehicle was traveling approximately 50 mph when it struck their vehicle. No airbags deployed.

At the moment of impact, Thomas reports that his body was jolted right to left. He denies any direct head trauma, but recalls momentary loss of consciousness and reports feeling dazed/confused for a sustained period of time after the accident. He relates that he had pain/discomfort involving the head, neck, upper back, low back and bilateral shoulders beginning within 30-60 mins of the collision and becoming progressively worse over the

following hours. A few days later, he started to develop headaches, dizziness, blurry vision, and brain fog.

Due to the severity of his injuries, he presented to the Chiropractor Dr. Zach Beatty, for further evaluation/management. A physical examination was performed and MRIs of the cervical and lumbar spine were ordered and scheduled for tomorrow. He continues to see the chiropractor twice weekly with slow, gradual relief.

MEDICAL HISTORY

CHRONIC MEDICAL CONDITIONS:

Sleep Apnea

SURGERIES:

Tonsilectomy during young childhood

ALLERGIES:

NKDA

CURRENT MEDICATIONS

None

PHYSICAL EXAMINATION

HEIGHT:

5'11"

WEIGHT:

234 pounds

BLOOD PRESSURE:

144/74 mm Hg, left arm sitting

PULSE:

73 beats per minute and regular

RESPIRATIONS:

17 breaths per minute

PULSE OXIMETRY:

96% on room air

GENERAL:

The patient appeared to be in **mild/moderate distress**. He ambulated without the use of an assistive device **but with mild difficulty noted while sitting on the examination table**

HEENT:

Normocephalic and atraumatic. PERRLA, EOMI b. No nystagmus noted b. b TMJs non tender to palpation with jaw opening/closing, no audible/palpable click noted. **Dizziness with extraocular eye movements. Dizziness with horizontal and vertical saccades**, but able to complete all 10 trials.

CERVICAL SPINE: Cervical flexion 25 degrees, extension 35 degrees. Left rotation 40 degrees, right rotation 30 degrees with tightness and pain reported in the bilateral paracervicals on end range of motion. Bilateral paracervicals with mild to moderate hypertrophy and tenderness to palpation.

THORACIC SPINE: Bilateral thoracolumbars with mild hypertrophy and tenderness to palpation.

LUMBAR SPINE: Forward flexion 70 degrees, extension 20 degrees with tightness and pain reported in the bilateral lumbosacrals on end range of motion. Bilateral lumbosacrals with moderate hypertrophy and tenderness to palpation.

EXTREMITIES: Bilateral shoulders with decreased range of motion, mild to moderate impingement, negative Neer and Hawkins, tightness and pain on end shoulder flexion in the bilateral paracervical muscles. Bilateral subacromial tenderness, right greater than left. Tenderness to palpation of the supraspinatus and infraspinatus region. Negative empty can test.

NEURO: Awake, alert and oriented x 3. **Positive Romberg w/ very mild sway on eyes closed.** 3/3 immediate object recall, **2/3 object recall after 5 mins.** Full sensation in upper and lower extremities to pinprick and light touch. +5/5 strength in upper/lower extremities bilaterally, 2/4 deep tendon reflexes bilaterally. Negative Hoffman's sign. **Mild gait ataxia with tandem heel toe walk.** No finger to nose dysmetria. Negative Spurling's test. Negative straight leg raises bilaterally.

ASSESSMENT

This is a 36 year old male, who was injured in a motor vehicle collision on 11/13/2024, sustaining injuries to his neck, mid/low back and bilateral shoulders. He also reports persistent mild/moderate headaches, dizziness, blurry vision, brain fog, and decreased memory, attention and concentration, anxiety and insomnia since his MVC.

V89.2XXA Person injured in motor vehicle traffic accident
S06.0 Concussion
S13.4xxA Sprain of ligament of cervical spine
S16.1xxA Strain of muscle, fascia and tendon at neck level
M50.90 Cervical disc disorder
S23.3xxA Sprain of ligament of thoracic spine

S29.012A Strain of muscle and tendon of back wall of thorax
S33.5XXA Sprain of ligaments of lumbar spine
S39.012A Strain of muscle, fascia and tendon of lower back
M51.86 Intervertebral disc displacement, lumbar region
M25.519 Pain in unspecified shoulder
S43.42 Sprain of rotator cuff capsule
S46.00 Injury of muscles and tendons of the rotator cuff
F43.22 Adjustment disorder with anxiety
F43.0 Acute stress reaction
F43.1 Post-traumatic stress disorder
G47.9 Sleep Disorder, unspecified
G47.01 Insomnia due to medical condition

TREATMENT PLAN

1. Headaches/Dizziness/Impaired Cognition – consistent with concussion given timeline of symptoms, positive Romberg, gait ataxia and cognitive impairment. **Consider ordering a head MRI w/ TBI protocol for further evaluation.** Cognitive rest, avoidance of EtOH advised. Strict monitoring/call back parameters discussed. NSAIDs, Magnesium supplementation and stress reduction techniques advised for post concussive headaches.
2. Neck pain - consistent with cervical spine strain, facet injury, possible disc injury and associated cervical muscle spasms. **Cervical spine MRI ordered and scheduled tomorrow.** Continue Chiropractic therapy sessions w/ Dr. Zach Beatty. **Scripts written for Diclofenac 75 mg q12h prn, lidocaine 4% ointment q12h prn and tizanidine 2 mg q12h prn for improved relief, sent electronically to Rite Aid in Sweetwater Road.** Heating pads prn and gentle home stretching advised. Should symptoms persist/worsen, may consider performing trigger point injections and/or submitting a referral for potential Epidural Steroid Injection to offer improved relief.
3. Mid/low back pain - consistent with lumbar spine strain, facet injury, possible disc injury and associated lumbosacral muscle spasms. **Lumbar spine MRI ordered and scheduled tomorrow.** Chiropractic care and pain management as outlined above. Should symptoms persist/worsen, may consider performing trigger point injections and/or submitting a referral for potential Epidural Steroid Injection to offer improved relief.
4. Bilateral shoulder pain – consistent with RTC tendinitis/impingement syndrome, possible RTC tear and labral tear. Chiropractic care and pain management as outlined above. Should symptoms persist/worsen, may consider ordering dedicated imaging and performing Cortisone vs PRP Injection for improved relief.

5. Anxiety/Adjustment Disorder – deep breathing/meditation, progressive muscle relaxation, guided imagery and desensitization/reexposure techniques discussed. Naturopathic supplementation with L-Theanine, GABA or Ashwagandha advised. Should the above measures fail, will consider prescribing psychotropic anxiolytic vs antidepressant medications for improved relief and referring to Psychologist for structured Cognitive Behavioral Therapy.
6. Insomnia – proper sleep hygiene measures discussed including setting consistent sleep schedule, shortening sleep time if laying awake in bed for sustained periods of time to strengthen association between time in bed with time asleep (sleep consolidation technique), minimize screen time/stress prior to bedtime and gentle stretching, light aerobic activity each day. Naturopathic supplementation as outlined above.

We will schedule the patient back into my clinic to be seen **on 1/9/2025 at 1:00 PM in my Mission Valley office** or earlier as needed for reevaluation.

DISCLAIMER/DISCLOSURE NOTE

I declare under the penalty of perjury that the name and qualification of each person who performed any service in connection with this report including diagnostic study and clerical preparation are as follows: Katelyn O'Connor, PA-C for history, physical examination, comprehensive evaluation and generation of this report to a meaningful form.

I declare under penalty of perjury that I personally performed the evaluation and physical examination of the patient or, in the case of a supplemental report, I personally performed the cognitive services necessary to produce the report and that, except as otherwise stated herein, the evaluation performed and the time spent performing the evaluation was in compliance with the guidelines.

By my signature on the report and this affidavit, I certify that this report represents the work product of myself and my staff in the manner described and expresses exclusively my professional opinion, findings, and conclusions on the matters discussed in the report.

Sincerely,

A handwritten signature in black ink, appearing to read "Katelyn O'Connor, PA-C". The signature is stylized and cursive.

Katelyn O'Connor, PA-C
NPI: 1982340048
CA License: PA61203

Case discussed with PA O'Connor, report reviewed and I am in agreement with assessment/plan as outlined above.

Sincerely,

A handwritten signature in black ink, appearing to read "Paul A. Jain, M.D., Q.M.E.", written in a cursive style.

PAUL A. JAIN, M.D., Q.M.E.

San Diego/San Marcos/Chula Vista/Murrieta/La Mesa/Santee

Diplomate, American Board of Internal Medicine

Diplomate, National Board of Physicians and Surgeons

Qualified Medical Examiner



353 W 9th Ave
Escondido, CA 92025

Phone: (619) 714-9100
Fax: (619) 363-4621
NPI: 1013647320
Tax ID: 88-1942212

RADIOLOGICAL CONSULTATION

Patient Name:	Peikert, Thomas	Study Date:	12/13/2024 13:40
Patient DOB:	03/09/1988	Study Name:	MR CERVICAL SPINE WO CONTRAST
Patient ID:	00018778	Referrer:	Beatty, Zach

FINAL REPORT

CLINICAL HISTORY: 36-year-old male presenting with neck pain, radiculopathy following trauma.

COMPARISON: None available at time of dictation.

TECHNIQUE: Multiplanar, multisequence images were acquired of the cervical spine without contrast.

FINDINGS:

There is reversal of the normal cervical lordosis. The atlantodental interval is maintained. The craniocervical junction is intact. The vertebral body heights are preserved. There is no fracture. The marrow signal is within normal limits. There is no cord signal abnormality.

C2-C3: There is no disc herniation, neuroforaminal narrowing or spinal canal stenosis.

C3-C4: There is a 1.3 mm central disc protrusion.

C4-C5: There is acute annular fissure/tear. There are 2.2 mm central and right foraminal and 1.6 mm left foraminal disc protrusions resulting in mild spinal canal stenosis, impingement of the exiting right nerve root and moderate right and mild left neural foraminal narrowing.

C5-C6: There is a large left foraminal annular rupture/tear with acute component and associated 4.9 mm disc protrusion resulting in impingement of the exiting left nerve root and severe left neural foraminal narrowing. Additional 3 mm central and 2 mm right foraminal disc protrusions resulting in abutment of the cord, moderate spinal canal stenosis, impingement of the exiting right nerve root and moderate right neural foraminal narrowing.

C6-C7: There is annular rupture/tear with acute component. There are 2.8 mm left paracentral and 2.4 mm right foraminal disc protrusion resulting in mild-to-moderate spinal canal stenosis and mild bilateral neural foraminal narrowing.

C7-T1: There is acute annular fissure/tear. There are 0.3 mm central and 2 mm bilateral foraminal disc protrusions resulting in mild bilateral neural foraminal narrowing.

IMPRESSION:

1. C5-C6: Large left foraminal annular rupture/tear with acute component and associated 4.9 mm disc protrusion resulting in impingement of the exiting left nerve root and severe left neural foraminal narrowing. Additional 3 mm central and 2 mm right foraminal disc protrusions resulting in abutment of the cord, moderate spinal canal stenosis, impingement of the exiting right nerve root and moderate right neural foraminal narrowing.
2. C4-C5: Acute annular fissure/tear. 2.2 mm central and right foraminal and 1.6 mm left foraminal disc protrusions resulting in mild spinal canal stenosis, impingement of the exiting right nerve root and moderate right and mild left



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RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 12/13/2024 13:40
Study Name: MR CERVICAL SPINE WO CONTRAST
Referrer: Beatty, Zach

neural foraminal narrowing.

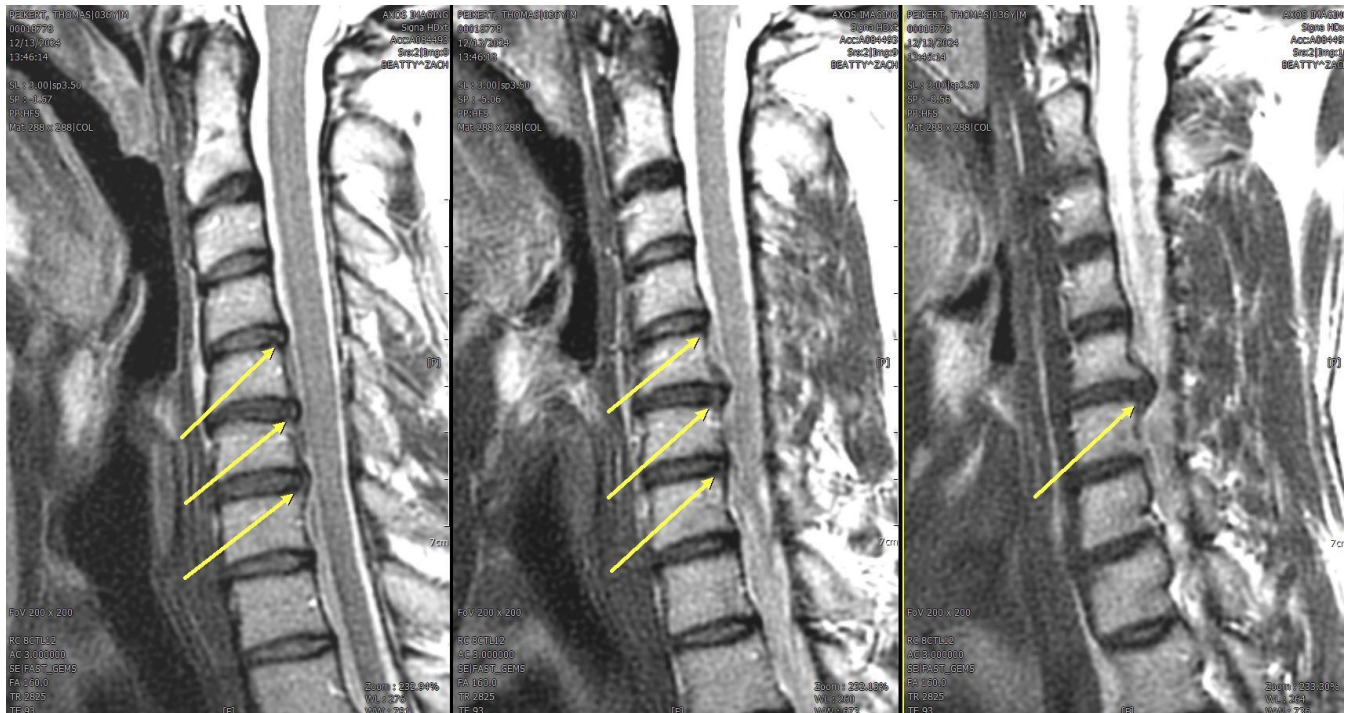
3. C6-C7: Annular rupture/tear with acute component. 2.8 mm left paracentral and 2.4 mm right foraminal disc protrusion resulting in mild-to-moderate spinal canal stenosis and mild bilateral neural foraminal narrowing.

Electronically signed by Radiologist, Wang, Michael , MD at 12/17/2024 10:55

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 12/13/2024 13:40
Study Name: MR CERVICAL SPINE WO CONTRAST
Referrer: Beatty, Zach





353 W 9th Ave
Escondido, CA 92025

Phone: (619) 714-9100
Fax: (619) 363-4621
NPI: 1013647320
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RADIOLOGICAL CONSULTATION

Patient Name:	Peikert, Thomas	Study Date:	12/13/2024 13:55
Patient DOB:	03/09/1988	Study Name:	MR LUMBAR SPINE WO CONTRAST
Patient ID:	00018778	Referrer:	Beatty, Zach

FINAL REPORT

CLINICAL HISTORY: 36-year-old male presenting with back pain, radiculopathy following trauma.

COMPARISON: None available at time of dictation.

TECHNIQUE: Multiplanar, multisequence images were acquired of the lumbar spine without contrast.

FINDINGS:

The vertebral body heights are preserved. The marrow signal is within normal limits. The visualized distal spinal cord and conus medullaris are within normal limits.

T12-L1: There is a 1.6 mm central disc protrusion.

L1-L2: There is 1.8 mm broad disc herniation.

L2-L3: There are 2.3 mm central and 2.6 mm bilateral foraminal disc protrusions resulting in mild bilateral neural foraminal narrowing.

L3-L4: There are 2.5 mm central and 3.3 mm bilateral foraminal disc protrusions resulting in abutment of the exiting nerve roots and mild-to-moderate bilateral neural foraminal narrowing.

L4-L5: There is annular rupture/tear. There are 4 mm central and 5 mm bilateral foraminal disc protrusions and facet arthrosis resulting in mild spinal canal stenosis, impingement of the exiting nerve roots and moderate bilateral neural foraminal narrowing.

L5-S1: There is annular rupture/tear. There are 3 mm central and 4 mm bilateral foraminal disc protrusions and facet arthrosis resulting in impingement of the exiting nerve roots and moderate bilateral neural foraminal narrowing.

IMPRESSION:

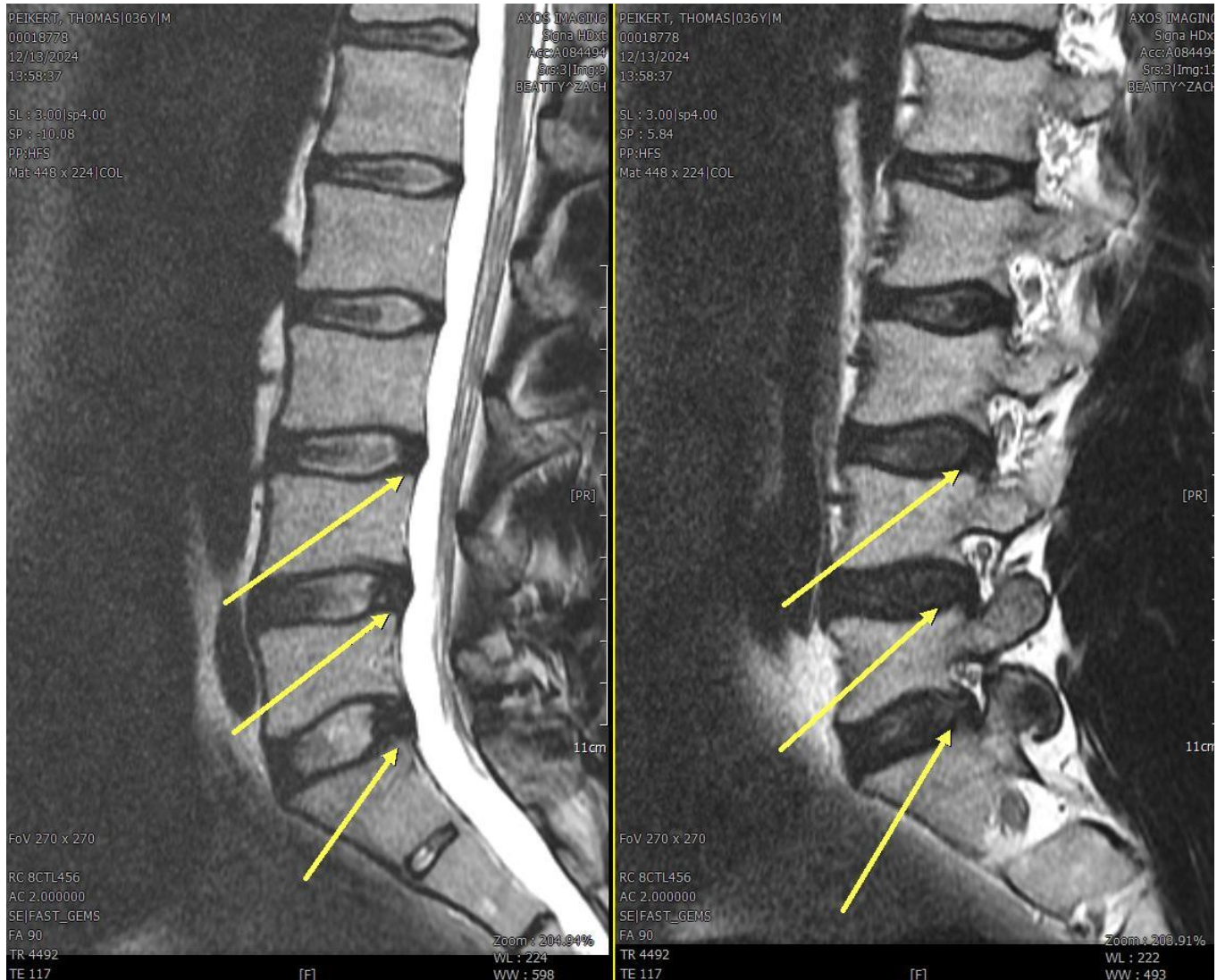
1. L4-L5: Annular rupture/tear. 4 mm central and 5 mm bilateral foraminal disc protrusions and facet arthrosis resulting in mild spinal canal stenosis, impingement of the exiting nerve roots and moderate bilateral neural foraminal narrowing.
2. L5-S1: Annular rupture/tear. 3 mm central and 4 mm bilateral foraminal disc protrusions and facet arthrosis resulting in impingement of the exiting nerve roots and moderate bilateral neural foraminal narrowing.
3. L3-L4: 2.5 mm central and 3.3 mm bilateral foraminal disc protrusions resulting in abutment of the exiting nerve roots and mild-to-moderate bilateral neural foraminal narrowing.

Electronically signed by Radiologist, Wang, Michael , MD at 12/17/2024 11:06

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 12/13/2024 13:55
Study Name: MR LUMBAR SPINE WO CONTRAST
Referrer: Beatty, Zach





353 W 9th Ave
Escondido, CA 92025

Phone: (619) 714-9100
Fax: (619) 363-4621
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RADIOLOGICAL CONSULTATION

Patient Name:	Peikert, Thomas	Study Date:	01/02/2025 09:36
Patient DOB:	03/09/1988	Study Name:	MR BRAIN WO CONTRAST (TBI)
Patient ID:	00018778	Referrer:	Jain, Paul

FINAL REPORT

CLINICAL HISTORY: 36-year-old male presenting following trauma, concussion.

TECHNIQUE: Multiplanar multisequence MRI of the brain was obtained without contrast on a 1.5 Tesla GE MRI (Signa HDXT 23x) with an 8-channel HD head coil. Traumatic Brain Injury/Concussion protocol was performed. DWI, T2, and FLAIR sequences were acquired as part of the routine brain MRI protocol.

Advanced neuroimaging techniques were performed including susceptibility-weighted imaging (SWI).

High-resolution sagittal T1-weighted images were obtained through the brain. Images were loaded into Cortechs lab software for postprocessing and comparative analysis with age and gender matched controls for volumetric analysis of 81 regions of the brain.

A DTI acquisition was obtained and processed on a separate workstation to evaluate white matter integrity with Fractional Anisotropy as the key metric. Data processing included visual quality control by our radiologist, eddy current correction, stripping of the skull data, tensor calculation, creation of scalar fractional anisotropy maps, tractography and whole-brain voxel-wise tract-based spatial statistics were analyzed. Standardized procedures based on open-source tools such as FSL (FMRIB Software Library v6.0, Oxford University) were used to calculate FA values from 48 fiber tracts using the Johns Hopkins University white matter atlas. Standardized statistical techniques were used to compare this patient to a group of controls. Data acquisition, processing and analysis were identical between the patient and control group.

COMPARISON: None available at time of dictation.

FINDINGS:

ROUTINE BRAIN MRI:

There is no restricted diffusion. There are approximately 10 foci of increased T2/FLAIR signal intensity within the bilateral frontal parietal centrum semiovale and corona radiata white matter. The ventricles are normal in caliber and morphology. The intracranial vessels demonstrate normal flow voids suggesting patency. There is mucosal thickening of the left maxillary sinus. The visualized mastoid air cells, remaining paranasal sinuses and orbits are within normal limits.

ADVANCED NEUROIMAGING TECHNIQUES:

- Susceptibility-weighted imaging (SWI): No susceptibility artifact is seen to suggest intracranial hemorrhage.
- Neuroquant Volumetric Analysis (Table 1):

The whole brain volume is in the 99th percentile compared to age and gender matched controls.

The cortical gray matter is in the 99th percentile compared to age and gender matched controls.



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Escondido, CA 92025

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Fax: (619) 363-4621
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RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 01/02/2025 09:36
Study Name: MR BRAIN WO CONTRAST (TBI)
Referrer: Jain, Paul

The cerebral white matter is in the 68th percentile compared to age and gender matched controls.

A complete description of the volumetric data is displayed in Table 1.

- Diffusion Tensor Imaging Findings (Tables 2 and 3):

Whole-brain voxel-based statistics were performed to analyze 48 white matter tracts from the Johns Hopkins University white matter atlas (Figure 1). Comparison was made to an age-matched control group of healthy volunteers.

2 out of 48 regions demonstrate decreased fractional anisotropy ($p < 0.05$), the right medial lemniscus and left fornix.

A complete description of the DTI and fractional anisotropy data is displayed in Table 2 and affected regional functions in Table 3.

IMPRESSION:

Routine MRI:

1. No macroscopic acute infarct or intracranial hemorrhage.
2. Approximately 10 foci of increased T2/FLAIR signal intensity within the bilateral frontal parietal centrum semiovale and corona radiata white matter representing white matter injury.

Advanced neuroimaging techniques:

Volumetric analysis demonstrates increased volume of the cortical gray matter in the 99th percentile compared to age and gender matched controls.

In the acute to sub-acute phase of traumatic brain injury a certain amount of edema/swelling can be expected to potentially increase structural volume. If damage is severe enough to cause neuronal death/shrinkage then over a period of months to years in the chronic setting relative atrophy/encephalomalacia can be seen resulting in decreased brain volume. However, increased volume can persist into the chronic phase in the setting of low-level inflammation.

DTI data indicate 2 out of 48 regions demonstrate decreased fractional anisotropy ($p < 0.05$), the right medial lemniscus and left fornix. These regions are involved in transmitting proprioceptive and sensory information, and memory and emotional processing.

Decreased fractional anisotropy (FA) values can be seen in disruption of the microstructural integrity of the white matter pathways and are believed to be a reflection of extracellular edema, demyelination, and/or axonal fiber loss. Possible etiologies of increased FA include a failure of developmental pruning, and/or disease/injury related intracellular cytogenic edema, neuroinflammation with microglial activation, astrogliosis and/or compensatory reorganization. Increased FA is most common during the acute to sub-acute phase following traumatic brain injury (<6 months),



353 W 9th Ave
Escondido, CA 92025

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RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 01/02/2025 09:36
Study Name: MR BRAIN WO CONTRAST (TBI)
Referrer: Jain, Paul

whereas decreased FA is most common during the chronic setting (>6 months) after injury. However, decreased FA values can be seen acutely, and in some cases, increased FA persists into the chronic period.

Volumetric and DTI results should be used in conjunction with a thorough medical history and physical examination including existing medical conditions, timeline of injury and symptoms, neurological and neuropsychological evaluation and laboratory findings.

Electronically signed by Radiologist, Wang, Michael , MD at 01/03/2025 23:26

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
 Patient DOB: 03/09/1988
 Patient ID: 00018778

Study Date: 01/02/2025 09:36
 Study Name: MR BRAIN WO CONTRAST (TBI)
 Referrer: Jain, Paul

Table 1: Neuroquant Volumetric Analysis

NeuroQuant® TBA

Triage Brain Atrophy Report: Single Timepoint

Patient Information

Patient: PEIKERT, THOMAS
 Referring Physician: JAIN, PAUL
 Age: 36 Sex: M Patient ID: 00018778

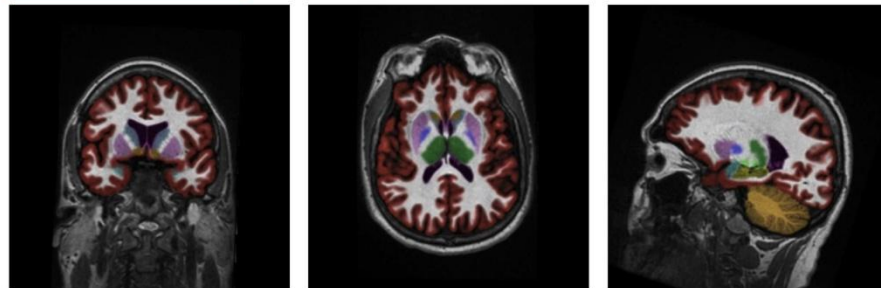
Report Information

Scan Date: 2025-01-02

Site Information

Axos Imaging
 (619) 714-9100

Brain Structure Visualization



Brain Structure Volumes

Brain Volumes	Volume (cm³)	Normative Percentile
Whole Brain	1271	99
Cortical Gray Matter	549	99
Cerebral White Matter	497	68
Cerebral WM Hypointensities*	0.08	—

Regional Brain Structures	Normative Percentiles		
	Left	Right	Total
Ventricles	89	89	90
Cerebellum	90	98	95
Brainstem	—	—	82
Deep Gray Nuclei			
— Putamen	10	14	11
— Thalamus	16	14	15
— Caudate	61	78	70
Cingulate	79	35	54
— Anterior Cingulate Gyrus	28	19	16
— Posterior Cingulate Gyrus	96	65	89

*White matter hypointensities are abnormally low signal intensity regions within the white matter as observed on a T1-weighted MRI scan.

Color Code Key:

A structure is below the 5th percentile OR a ventricle is above the 95th percentile.
 A structure is above the 95th percentile OR a ventricle is below the 5th percentile

Regional Lobe Analysis	Normative Percentiles		
	Left	Right	Total
Temporal Lobe	93	98	97
— Hippocampus	49	63	56
— Entorhinal Cortex	34	52	40
— Parahippocampal Gyrus	61	53	59
— Amygdala	86	55	76
— Fusiform Gyrus	19	54	32
— Superior Temporal Gyrus	97	89	97
— Middle Temporal Gyrus	98	97	99
— Inferior Temporal Gyrus	86	97	94
— Temporal Pole	36	91	70
Parietal Lobe	99	99	99
— Primary Sensory Cortex	80	47	66
— Superior Parietal Lobule	99	99	99
— Supramarginal Gyrus	40	99	81
Frontal Lobe	99	99	99
— Primary Motor Cortex	70	45	59
— Superior Frontal Gyrus	98	99	99
— Middle Frontal Gyrus	99	99	99
— Inferior Frontal Gyrus	85	93	94
— Lateral Orbitofrontal Gyrus	99	98	99
— Medial Orbitofrontal Gyrus	94	95	97
Occipital Lobe	27	46	35
— Medial Occipital	2	22	7
— Lateral Occipital	83	79	84

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas

Patient DOB: 03/09/1988

Patient ID: 00018778

Study Date: 01/02/2025 09:36

Study Name: MR BRAIN WO CONTRAST (TBI)

Referrer: Jain, Paul

Table 2: Fractional Anisotropy Values

Decreased FA
 Increased FA

Patient: PEIKERT, THOMAS Age: 36 Patient ID: 00018778 Scan Date: 01/02/2025					
Num	Region	Patient	Controls	p-value	
01	Middle cerebellar peduncle	0.485	0.462	NS	
02	Pontine crossing tract (a part of MCP)	0.485	0.473	NS	
03	Genu of corpus callosum	0.634	0.645	NS	
04	Body of corpus callosum	0.496	0.513	NS	
05	Splenium of corpus callosum	0.727	0.711	NS	
06	Fornix (column and body of fornix)	0.377	0.411	NS	
07	Corticospinal tract R	0.591	0.542	NS	
08	Corticospinal tract L	0.603	0.563	NS	
09	Medial lemniscus R	0.484	0.554	0.025	
10	Medial lemniscus L	0.518	0.553	NS	
11	Inferior cerebellar peduncle R	0.448	0.506	0.100	
12	Inferior cerebellar peduncle L	0.460	0.499	NS	
13	Superior cerebellar peduncle R	0.536	0.550	NS	
14	Superior cerebellar peduncle L	0.574	0.572	NS	
15	Cerebral peduncle R	0.609	0.610	NS	
16	Cerebral peduncle L	0.604	0.626	NS	
17	Anterior limb of internal capsule R	0.529	0.516	NS	
18	Anterior limb of internal capsule L	0.518	0.508	NS	
19	Posterior limb of internal capsule R	0.652	0.644	NS	
20	Posterior limb of internal capsule L	0.675	0.651	NS	
21	Retrolenticular part of internal capsule R	0.506	0.526	NS	
22	Retrolenticular part of internal capsule L	0.516	0.536	NS	
23	Anterior corona radiata R	0.438	0.462	NS	
24	Anterior corona radiata L	0.436	0.459	NS	
25	Superior corona radiata R	0.444	0.444	NS	
26	Superior corona radiata L	0.457	0.457	NS	
27	Posterior corona radiata R	0.435	0.469	NS	
28	Posterior corona radiata L	0.427	0.447	NS	
29	Posterior thalamic radiation (include optic radiation) R	0.571	0.564	NS	
30	Posterior thalamic radiation (include optic radiation) L	0.525	0.558	NS	
31	Sagittal stratum (incl inferior longitudinal fasciculus and inferior fronto-occipital fasciculus) R	0.474	0.481	NS	
32	Sagittal stratum (incl inferior longitudinal fasciculus and inferior fronto-occipital fasciculus) L	0.473	0.474	NS	
33	External capsule R	0.404	0.414	NS	
34	External capsule L	0.422	0.414	NS	
35	Cingulum (cingulate gyrus) R	0.412	0.404	NS	
36	Cingulum (cingulate gyrus) L	0.465	0.436	NS	
37	Cingulum (hippocampus) R	0.400	0.422	NS	
38	Cingulum (hippocampus) L	0.376	0.420	NS	
39	Fornix (cres) / Stria terminalis R	0.430	0.462	NS	
40	Fornix (cres) / Stria terminalis L	0.401	0.470	0.025	
41	Superior longitudinal fasciculus R	0.440	0.434	NS	
42	Superior longitudinal fasciculus L	0.430	0.444	NS	
43	Superior fronto-occipital fasciculus R	0.498	0.467	NS	
44	Superior fronto-occipital fasciculus L	0.495	0.460	NS	
45	Uncinate fasciculus R	0.499	0.519	NS	
46	Uncinate fasciculus L	0.444	0.466	NS	
47	Tapetum R	0.407	0.484	NS	
48	Tapetum L	0.470	0.493	NS	

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
 Patient DOB: 03/09/1988
 Patient ID: 00018778

Study Date: 01/02/2025 09:36
 Study Name: MR BRAIN WO CONTRAST (TBI)
 Referrer: Jain, Paul

Table 3: White Matter Tract Regional Functions
 Decreased FA
 Increased FA

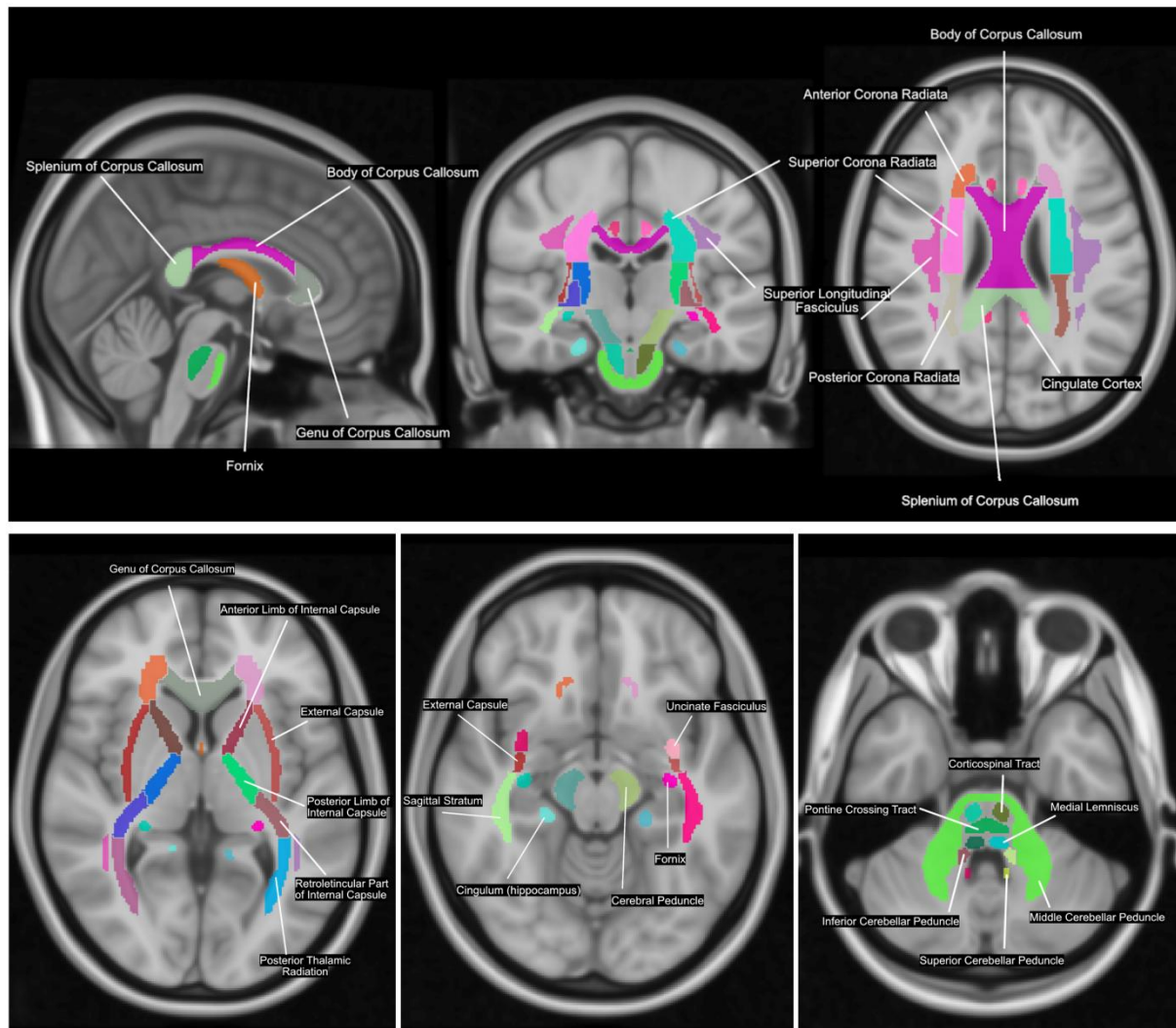
Patient: PEIKERT, THOMAS Age: 36 Patient ID: 00018778 Scan Date: 01/02/2025		
Regions	Connections	Function
Middle Cerebral Peduncle	Connects Cerebellum to the Pons	Initiation and Timing of Volitional Movement
Pontine Crossing Tracts	Connects Pons and Contralateral Cerebellum	Coordination of Movement
Genu of the Corpus Callosum	Connects Right and Left Anterior Frontal Lobes	Interhemispheric Integration of Executive Functions
Body of the Corpus Callosum	Connects Right and Left Posterior Frontal Lobes Connects Right and Left Parietal Lobes	Interhemispheric Integration of Motor and Somatosensory Functions
Splenium of the Corpus Callosum	Connects Right and Left Occipital Lobes	Visuospatial Information Transfer
Fornix	Connects Hippocampus and Septal Region to the Hypothalamus	Formation of Short-Term Memory
Corticospinal Tracts	Connects Primary Motor Cortex with Contralateral Spinal Motor Neurons	Voluntary Motor Control
Medial Lemniscus	Connects Dorsal Column Nuclei with the Contralateral Thalamus (VPL)	Transmit Proprioceptive and Sensory Information
Inferior Cerebellar Peduncles	Connects Spinal Cord and the Medulla to the Cerebellum	Integration of Proprioception with Motor Functions for Balance and Posture Maintenance
Superior Cerebellar Peduncles	Connects Cerebellum to Pons and Midbrain	Motor Coordination and Balance
Cerebral Peduncles	Connects Cerebellum with the Thalamus and Motor Cortex	New Motor Skills, Refining Motor Skills, Proprioception
Anterior Limb of the Internal Capsule	Contains Fibers Interconnecting the Thalamus and Frontal Lobe; Lentiform and Caudate Nuclei; Cortex and Corpus Striatum	Emotion, Motivation, Cognitive Processing, Decision-Making
Posterior Limb of the Internal Capsule	Contains Fibers Interconnecting Motor Areas with the Brainstem; Midbrain and the Thalamus, Occipital Lobes, and Temporal Lobes	Motor Function, Visual-Spatial Processing, Visual Motion, Face and Word Processing (L/R)
Retrolenticular Part of the Internal Capsule	Connects Thalamus and Occipital Cortex	Visual Processing
Anterior Corona Radiata	Contains Descending and Ascending Fibers Related to Frontal Lobe Cortex	Executive Function, Emotional Control
Superior Corona Radiata	Contains Descending and Ascending Fibers Related to Motor Cortex	Motor Control
Posterior Corona Radiata	Contains Descending and Ascending Fibers Related to Parietal Lobe Cortex	Attention Control, Somatosensory Function
Posterior Thalamic Radiation	Connects Thalamus and Cortex	Visual and Auditory Transmission
Sagittal Stratum	Connects Thalamus with Occipital, Parietal, Temporal and Cingulate Cortices	Verbal and Non-Verbal Language Processing, Reading and Visual Recognition
External Capsule	Contains Cortico-Cortical Association Fibers for Occipital, Temporal, Parietal, and Cingulate Cortices	Cognitive Processing
Cingulum (cingulate cortex)	Connects Cingulate and Pre-Frontal Cortices	Emotional, Sensory Processing and Decision Making
Cingulum (hippocampus)	Connects Cingulate and Entorhinal Cortices	Memory, Emotional Processing
Fornix (cres) and Stria Terminalis	Connects Hippocampus and Mammillary Bodies; Amygdala with the Septal Region and Hypothalamus	Memory Formation, Emotional Processing, Fear Response
Superior Longitudinal Fasciculus	Connects the Frontotemporal and Frontoparietal Regions	Integration of Auditory and Speech Nuclei
Superior Fronto-Occipital Fasciculus	Connects the Frontal Lobe with the Ipsilateral Parietal Lobe (name is misnomer)	Spatial Awareness, Symmetric Processing
Uncinate Fasciculus	Connects the Limbic System with the Orbitofrontal Cortex	Auditory Declarative and Verbal Memory, Emotional Processing
Tapetum	Contains Commissural Fibers Interconnecting Right and Left Temporal Lobes	Interhemispheric Integration of Auditory Processing

RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

Study Date: 01/02/2025 09:36
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Figure 1: Johns Hopkins University White Matter Atlas superimposed upon the Montreal Neurological Institute Standard Brain Template.



RADIOLOGICAL CONSULTATION

Patient Name: Peikert, Thomas
Patient DOB: 03/09/1988
Patient ID: 00018778

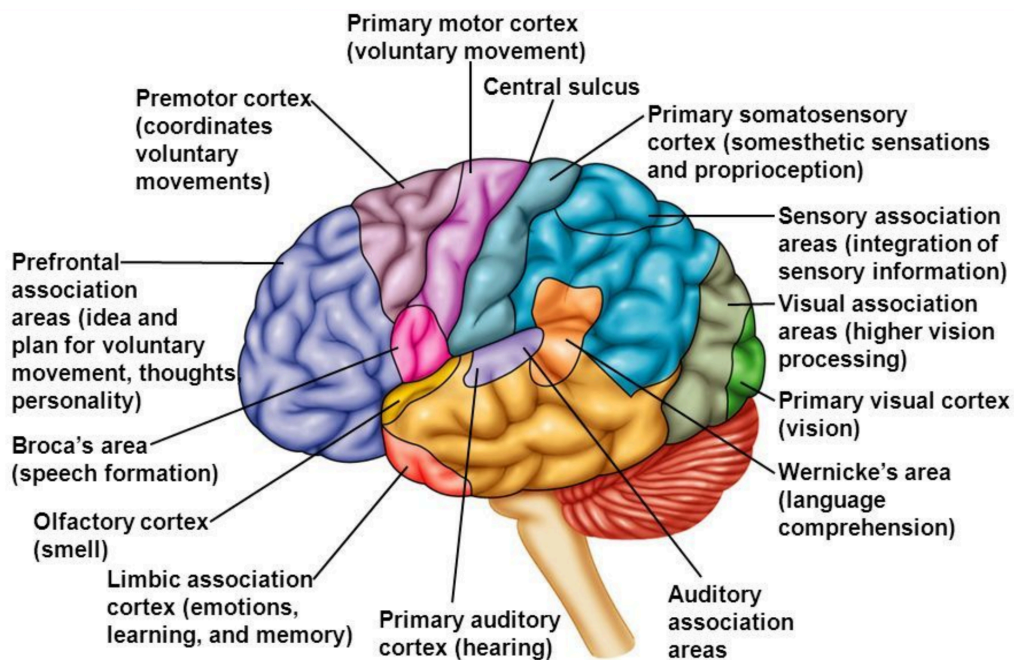
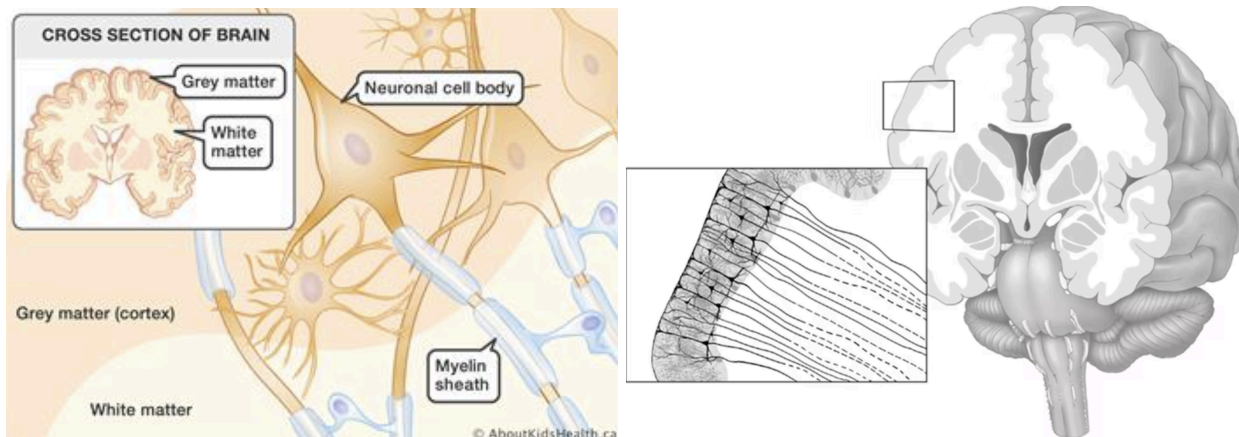
Study Date: 01/02/2025 09:36
Study Name: MR BRAIN WO CONTRAST (TBI)
Referrer: Jain, Paul



MRI TBI Supplement

Basic Neuroanatomy:

There are an estimated 86 billion neurons in the human brain. The neuron cell bodies are typically located in the gray matter of the brain and the axons carry signals as myelinated white matter tracts. Different regions of the brain are responsible for different functions including motor skills, sensation, behavior, language, emotion and decision-making.

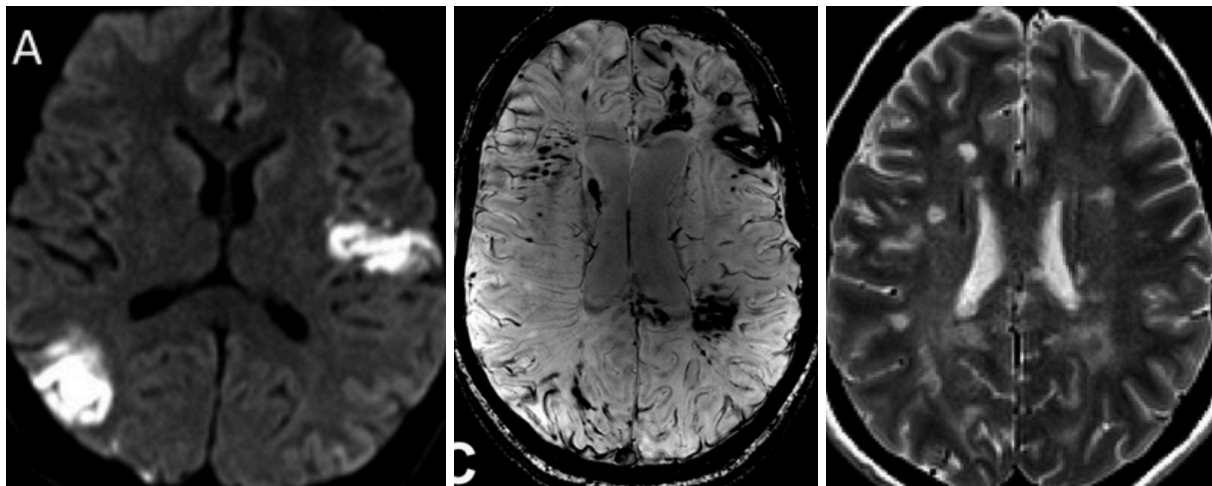


Magnetic Resonance Imaging:

MRI utilizes a strong magnetic field and radiofrequency waves to evaluate hydrogen atom behavior in living tissue. DWI (diffusion-weighted imaging) evaluates for acute stroke. T2/FLAIR evaluates for white matter lesions and brain swelling/edema. SWI (susceptibility-weighted imaging) is the most sensitive means of detecting hemorrhage. DTI (diffusion tensor imaging) evaluates the structural integrity of white matter tracts or axons.

In moderate/severe trauma and occasionally mild traumatic brain injury, findings are typically present on routine imaging sequences, however, in many cases of mild TBI only advanced neuroimaging techniques such as volumetric analysis or DTI will detect abnormalities.

Routing Imaging Sequences:

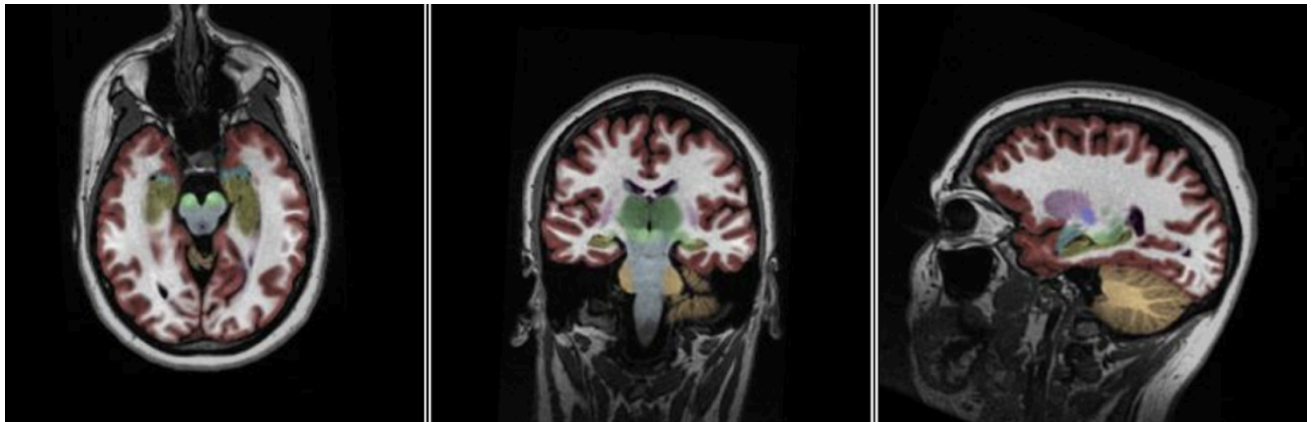


Volumetric Analysis:

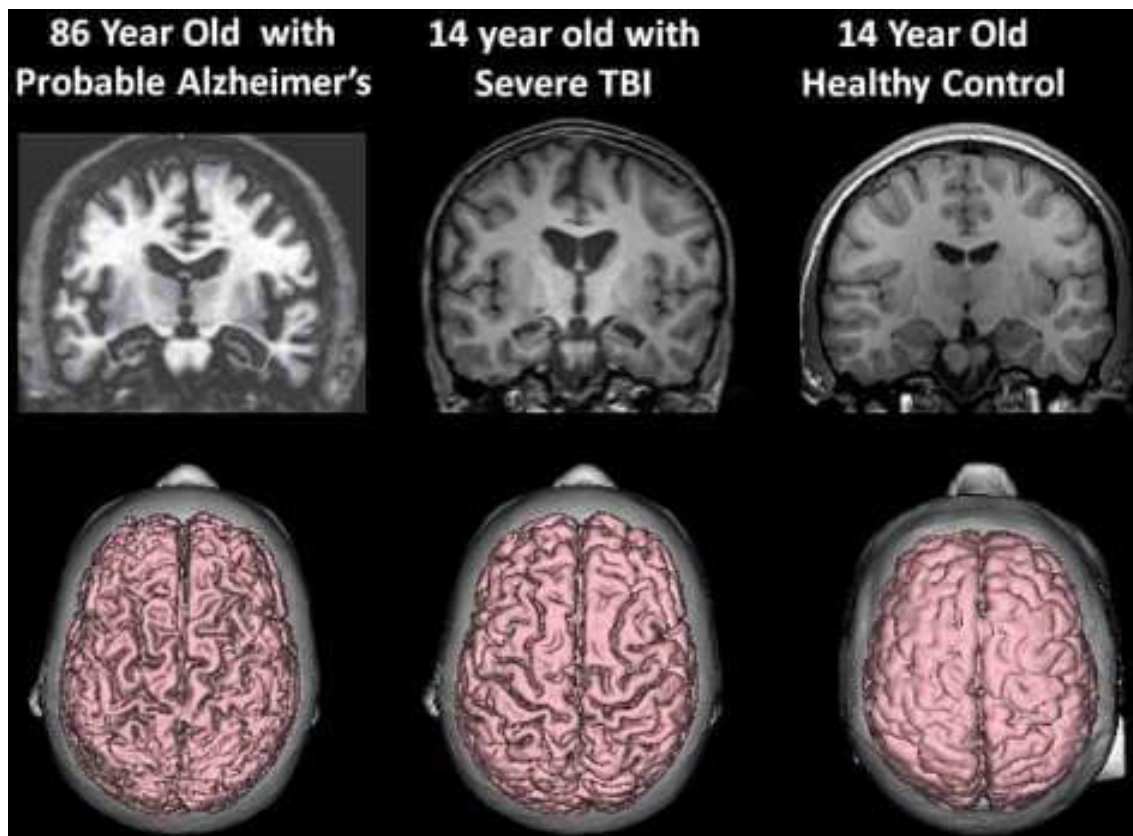
Brain volume is often altered by trauma. In acute trauma brain volume typically increases and in the chronic phase atrophy occurs resulting in decreased volume. Visual evaluation of volume in routine MRI images can be subjective and subtle findings can be overlooked.

Volumetric analysis improves upon these issues by utilizing computer algorithms to process high-resolution 3D images of the brain and then automatically calculate the volume of the different brain regions and compare the patient's volumes to expected volumes from healthy brains.

We utilize CorTechs Labs' Neuroquant 3.0 which compares our patient to several thousand controls in their normative database. 81 different brain regions are evaluated.

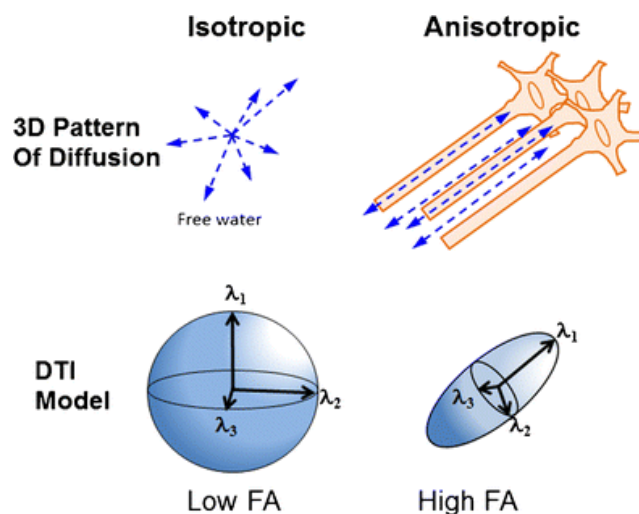


Examples of normal brain volume and brain volume affected by trauma are below.



Diffusion Tensor Imaging:

Diffusion tensor imaging evaluates the microstructural integrity of axons or white matter tracts by describing fractional anisotropy (FA), the behavior and motion of water molecules in the brain. White matter tracts are typically bundled fibers and as such water molecules can be expected to flow in certain directions along these fibers like pipes in a building. When there is disruption of the white matter tracts water molecules tend to flow more freely in random directions. When all water molecules are traveling in a single direction FA is 1. When there is complete freedom of motion in any and all directions FA is 0.



Decreased fractional anisotropy (FA) values can be seen in disruption of the integrity of the white matter pathways and are believed to be a reflection of extracellular edema, demyelination, and/or axonal fiber loss. Possible etiologies of increased FA include a failure of developmental pruning, and/or disease/injury related intracellular cytogenic edema, neuroinflammation with microglial activation, astrogliosis and/or compensatory reorganization. Increased FA is most common during the acute to sub-acute phase following traumatic brain injury (<6 months), whereas decreased FA is most common during the chronic setting (>6 months) after injury. However, decreased FA values can be seen acutely, and in some cases, increased FA persists into the chronic period.

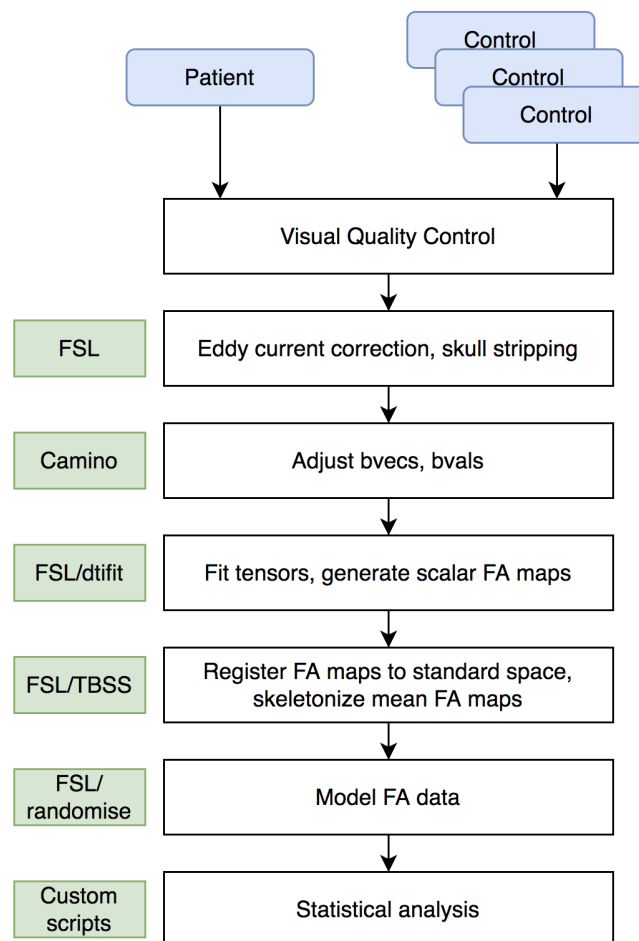
We utilize standardized and automated processes based on FSL (FMRIB Software Library v6.0, Oxford University) to calculate FA values from 48 fiber tracts using the Johns Hopkins University white matter atlas. Over 10 different tools are used in our DTI processing pipeline.

Data processing includes visual quality control by our radiologist, eddy current correction, stripping of the skull data and tensor calculation. Scalar fractional anisotropy maps are created and whole-brain voxel-wise tract-based spatial statistics are performed using a threshold-free cluster enhancement (TFCE) method with family-wise error (FWE) correction for multiple comparisons.

Standardized statistical techniques including unpaired two-sample t-test are used to compare this patient to a group of controls. Data acquisition, processing and analysis are identical between the patient and control group.

Our DTI processing pipeline is outlined below.

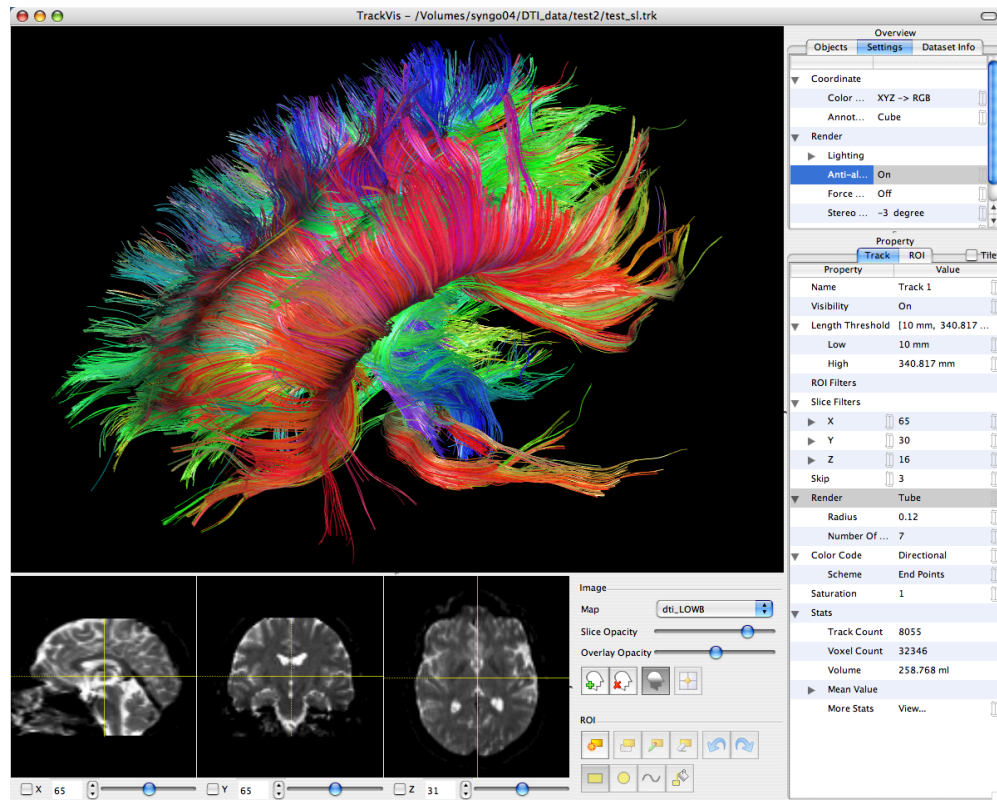
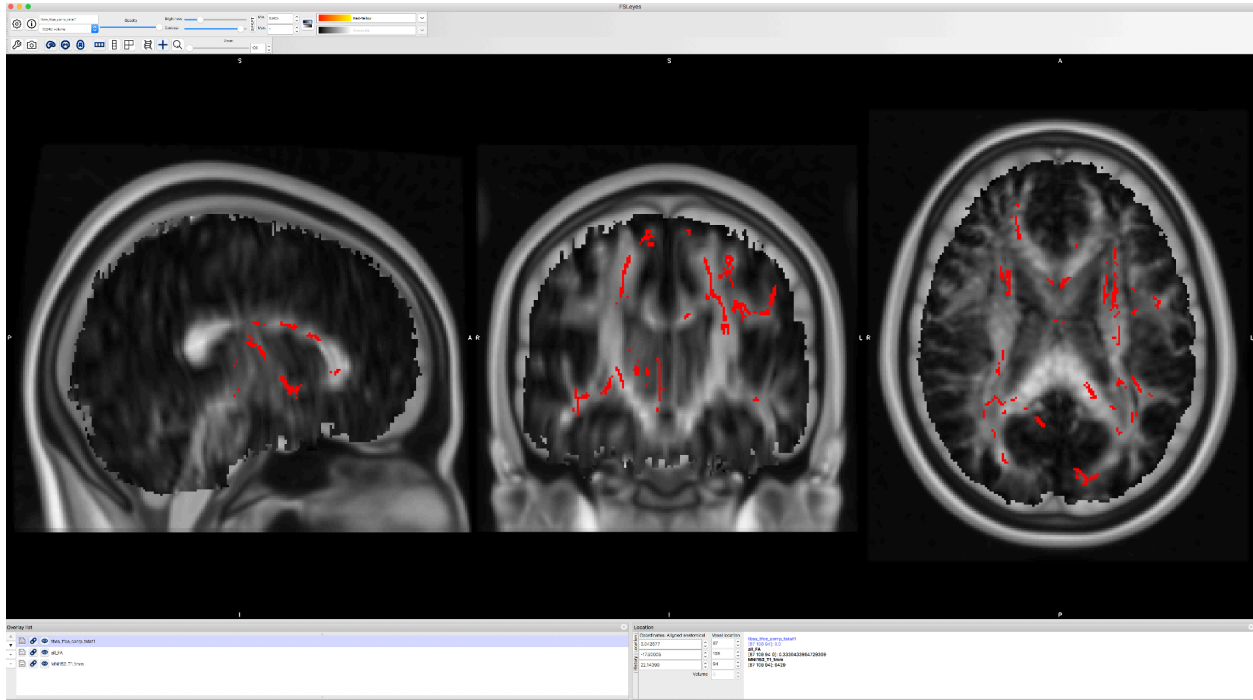
DTI Processing Pipeline





353 W 9th Ave
Escondido, CA 92025
(619) 714-9100
www.axosimaging.com

```
===== dcm2niix =====
Chris Rorden's dcm2niix version v1.0.20190902 (JP2:OpenJPEG) (JP-LS:CharLS) Clang8.1.0
Found 598 DICOM file(s)
Image Decompression is new: please validate conversions
Warning: Guessing slice times using ProtocolBlock SliceOrder=1 (seq=0, int=1)
Convert 598 DICOM as ./_DTI_20220421131233_9 (256x256x23x26)
Conversion required 2.752569 seconds (2.599506 for core code).
===== dcm2niix Complete =====
===== Renaming =====
===== Renaming Complete =====
===== eddy_correct =====
processing DTI_eddy_tmp0000
processing DTI_eddy_tmp0001
processing DTI_eddy_tmp0002
processing DTI_eddy_tmp0003
processing DTI_eddy_tmp0004
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processing DTI_eddy_tmp0021
processing DTI_eddy_tmp0022
processing DTI_eddy_tmp0023
processing DTI_eddy_tmp0024
processing DTI_eddy_tmp0025
===== eddy_correct Complete =====
===== Skull Stripping =====
===== Skull Stripping Complete =====
===== Rotate bvecs =====
Generating ... DTI_eddy_tmp0000.mat
Generating ... DTI_eddy_tmp0001.mat
Generating ... DTI_eddy_tmp0002.mat
Generating ... DTI_eddy_tmp0003.mat
Generating ... DTI_eddy_tmp0004.mat
Generating ... DTI_eddy_tmp0005.mat
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Generating ... DTI_eddy_tmp0017.mat
Generating ... DTI_eddy_tmp0018.mat
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PAUL A. JAIN, M.D., Q.M.E.
3838 Camino Del Rio N, STE 240
San Diego, CA 92108
(619) 880-8844

Other locations: San Marcos, Chula Vista, Murrieta, La Mesa, Santee

*Diplomate, American Board of
Internal Medicine*

*Diplomate, National Board of
Physicians and Surgeons*

Qualified Medical Examiner

January 9, 2025

RE: Peikert, Thomas
D/B: 03/9/1988
D/I: 11/13/2024
D/E: 01/9/2025

FOLLOW UP INTERNAL MEDICINE REPORT

Mr. Thomas Peikert, a construction worker, reports persistent soreness in the left shoulder, upper back, and lower back since his accident. Initially, symptoms improved rapidly but have since plateaued with ongoing soreness. He reports difficulty sleeping due to pain, daily headaches ranging from mild to moderate, blurred vision, brain fog, and short-term memory lapses, which significantly affect his work performance in both construction and clerical tasks.

Chiropractic care with Dr. Zach Beatty has been beneficial, with a transition to light adjustments. His current medications include Diclofenac, Tizanidine, and lidocaine ointment, providing moderate, temporary relief.

HISTORY OF PRESENT ILLNESS

Please refer to the initial report for full HPI.

MEDICAL HISTORY

CHRONIC MEDICAL
CONDITIONS:

Sleep Apnea

SURGERIES:

Tonsillectomy during young childhood

ALLERGIES:

NKDA

CURRENT MEDICATIONS

Diclofenac 75 mg q12h prn
Lidocaine 4% ointment q12h prn
Tizanidine 2 mg q12h prn

PHYSICAL EXAMINATION

HEIGHT: 5'11"

WEIGHT: 232 pounds

BLOOD PRESSURE: 138/78 mmHg, left arm sitting

PULSE: 69 beats per minute and regular

RESPIRATIONS: 17 breaths per minute

PULSE OXIMETRY: 95% on room air

GENERAL: The patient appeared to be in **mild/moderate distress**. He ambulated without the use of an assistive device but with **mild difficulty noted while sitting on the examination table**.

HEENT: Normocephalic and atraumatic. PERRLA, EOMI b. No nystagmus noted b. b TMJs non tender to palpation with jaw opening/closing, no audible/palpable click noted. **Dizziness, lightheadedness, and blurry vision with extraocular movements. Dizziness and lightheadedness with horizontal/vertical saccades**, but able to complete all 10 trials.

CERVICAL SPINE: **Cervical flexion 40 degrees, extension 30 degrees. Left lateral rotation 50 degrees, right lateral rotation 50 degrees with tightness and pain in the bilateral paracervical muscles, right greater than left. Bilateral paracervical myofascial trigger point spasming, right greater than left.**

THORACIC SPINE: **Bilateral rhomboids myofascial trigger point spasming, right greater than left.**

LUMBAR SPINE: **Forward flexion 40 degrees, extension 20 degrees with tightness and pain noted in the lumbosacral muscles. Bilateral lumbosacral**

myofascial trigger point spasming, hypertrophy, tenderness to palpation, right greater than left.

NEURO:

Awake, alert, and oriented x 3. **Positive Romberg w/ moderate sway with eyes closed.** Full sensation in upper and lower extremities to pinprick and light touch. +5/5 strength in upper and lower extremities bilaterally, 2/4 deep tendon reflexes bilaterally. **Moderate gait ataxia with tandem heel toe walk.** No finger to nose dysmetria. Negative Spurling's bilaterally. Negative straight leg raises bilaterally.

DIAGNOSTICS

Head MRI with TBI protocol on 1/5/2025 at Axos Imaging

TRIGGER POINT INJECTION PROCEDURE NOTE

Informed consent discussed regarding risks/benefits of Trigger Point Injections including potential for bleeding, bruising and/or infection. Patient verbalized full understanding of risks/benefits and agreed to proceed with TPIs at clinic bedside. Injection sites cleansed with alcohol swabs and **Trigger point injections x 2 performed to the bilateral paracervical muscles** using 25 g 1" needle with 1 cc mixture of 3 cc Bupivacaine 0.5% and 0.5 cc Kenalog 40 mg/ml injected into each trigger point site with good immediate relief and no complications noted. Additional **Trigger point injections x 2 performed to bilateral lumbosacral muscles** using 25 g 1" needle with 1 cc mixture of 3 cc Bupivacaine 0.5% and 0.5 cc Kenalog 40 mg/ml injected into each trigger point site with good immediate relief and no complications noted

Each injection site/trigger point massaged with focal acupressure for 1 minute and sites bandaged with no post procedural complications noted. Prior to completion of appointment, patient reported moderate relief of his trigger point discomfort with good anesthetic effect at each injection site.

Home care instructions advised including keeping injection sites clean & dry, avoiding applying direct heat to injection sites and leaving bandages on for 12 hours. Call back parameters discussed.

ASSESSMENT

This is a 36-year-old male, who was injured in a motor vehicle collision on 11/13/2024, sustaining injuries to his neck, mid/low back and bilateral shoulders. He also reports persistent mild/moderate headaches, dizziness, blurry vision, brain fog, and decreased memory, attention and concentration, anxiety and insomnia since his MVC.

V89.2XXA Person injured in motor vehicle traffic accident
S06.0 Concussion
S13.4xxA Sprain of ligament of cervical spine
S16.1xxA Strain of muscle, fascia and tendon at neck level
M50.90 Cervical disc disorder
S23.3xxA Sprain of ligament of thoracic spine
S29.012A Strain of muscle and tendon of back wall of thorax
S33.5XXA Sprain of ligaments of lumbar spine
S39.012A Strain of muscle, fascia and tendon of lower back
M51.86 Intervertebral disc displacement, lumbar region
M25.519 Pain in unspecified shoulder
S43.42 Sprain of rotator cuff capsule
S46.00 Injury of muscles and tendons of the rotator cuff
F43.22 Adjustment disorder with anxiety
F43.0 Acute stress reaction
G47.9 Sleep Disorder, unspecified
G47.01 Insomnia due to medical condition

TREATMENT PLAN

1. Headaches/Dizziness/Impaired Cognition – **Head MRI with TBI protocol reviewed. Notable findings include y 10 foci of increased T2/FLAIR signal intensity within the bilateral frontal parietal centrum semiovale and corona radiata white matter representing white matter injury. Volumetric analysis demonstrates increased volume of the cortical gray matter in the 99th percentile compared to age and gender-matched controls. DTI data indicate 2 out of 48 regions demonstrate decreased fractional anisotropy ($p<0.05$), the right medial lemniscus and left fornix. These regions are involved in transmitting proprioceptive and sensory information, and memory and emotional processing.**

Cognitive rest, avoidance of EtOH advised. Strict monitoring/call back parameters discussed. NSAIDs, Magnesium supplementation, and stress reduction techniques advised for post concussive headaches.

2. Neck pain - **Cervical spine MRI previously completed, official radiology report requested.** Continue current Chiropractic Care with Dr. Zach Beatty. Continue current prescribed analgesics regimen. **TPIs x 2 to the bilateral paracervical muscles given today with good relief and no complications noted.**
3. Upper/lower back pain - **Lumbar spine MRI previously completed, official radiology report requested.** Continue Chiropractic Care, Dr. Zach Beatty. Continue pain

management as outlined above. **TPIs x 2 to the bilateral lumbosacral muscles given today with good relief and no complications noted.**

4. Bilateral shoulder pain – consistent with RTC tendinitis/impingement syndrome, possible RTC tear and labral tear. Chiropractic care and pain management as outlined above. Should symptoms persist/worsen, may consider ordering dedicated imaging and performing Cortisone vs PRP Injection for improved relief.
5. Anxiety/Adjustment Disorder – deep breathing/meditation, progressive muscle relaxation, guided imagery and desensitization/re-exposure techniques discussed. Naturopathic supplementation with L-Theanine, GABA or Ashwagandha advised if still symptomatic. Should the above measures fail, will consider prescribing psychotropic anxiolytic vs antidepressant medications for improved relief and referring to Psychologist for structured Cognitive Behavioral Therapy.
6. Insomnia – proper sleep hygiene measures discussed including setting consistent sleep schedule, shortening sleep time if laying awake in bed for sustained periods of time to strengthen association between time in bed with time asleep (sleep consolidation technique), minimize screen time/stress prior to bedtime and gentle stretching, light aerobic activity each day. Naturopathic supplementation as outlined above.

We will schedule the patient back into my clinic to be seen **on February 6th at 10:00 AM in Mission Valley** or earlier as needed for reevaluation.

Sincerely,



PAUL A. JAIN, M.D., Q.M.E.

**San Diego/San Marcos/Chula Vista/Murrieta/La Mesa/Santa Ana
Diplomate, American Board of Internal Medicine
Diplomate, National Board of Physicians and Surgeons
Qualified Medical Examiner**

PAUL A. JAIN, M.D., Q.M.E.

3838 Camino Del Rio N, STE 240

San Diego, CA 92108

(619) 880-8844

Other locations: San Marcos, Chula Vista, Murrieta, La Mesa, Santee

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Physicians and Surgeons*

Qualified Medical Examiner

March 6, 2025

RE: Peikert, Thomas
D/B: 03/9/1988
D/I: 11/13/2024
D/E: 03/06/2025

FOLLOW UP INTERNAL MEDICINE REPORT

The patient reports persistent tension and soreness in his shoulders and upper back, extending to the back of his neck and up to his head, turning into mild to moderate headaches. He feels tender in the back of his head and neck as well. These headaches occur about 3-4 times throughout the week, accompanied by blurry vision and nausea when severe. Although the blurry vision comes and goes randomly. Brain fog, lightheadedness, and dizziness still persist but are not as severe as before.

Thomas says that his left shoulder feels worse than his right and he feels restricted when lifting or reaching overhead. He avoids carrying anything heavier than 30 pounds. Lower back pain comes and goes, ranging from mild to moderate, and occasionally severe. He constantly feels tense in his midback and lower back.

He continues to do light work duty and continues to see Dr. Beatty twice weekly for chiropractic care which offers moderate relief.

HISTORY OF PRESENT ILLNESS

Please refer to the initial report for full HPI.

MEDICAL HISTORY

CHRONIC MEDICAL
CONDITIONS:

Sleep Apnea

SURGERIES:

Tonsillectomy during young childhood

ALLERGIES:

NKDA

No other medical conditions reported, will review should new medical records be available.

CURRENT MEDICATIONS

Diclofenac 75 mg q12h prn

Lidocaine 4% ointment q12h prn

Tizanidine 2 mg q12h prn

PHYSICAL EXAMINATION

HEIGHT: 5'11"

WEIGHT: 232 pounds

BLOOD PRESSURE: 128/76 mmHg, left arm sitting

PULSE: 75 beats per minute and regular

RESPIRATIONS: 17 breaths per minute

PULSE OXIMETRY: 97% on room air

GENERAL: The patient appeared to be in **mild distress**. He ambulated without the use of an assistive device but with no difficulty noted while sitting on the examination table.

HEENT: Normocephalic and atraumatic. PERRLA, EOMI b. No nystagmus noted b. b TMJs non tender to palpation with jaw opening/closing, no audible/palpable click noted. **Mild dizziness, lightheadedness, and blurry vision with extraocular movements. Dizziness and lightheadedness with horizontal/vertical saccades**, but able to complete all 10 trials. Visual Acuity test: **Left eye 20/25. Right 20/40. Both 20/30.**

CERVICAL SPINE: Cervical flexion 40 degrees, extension 30 degrees. Left lateral rotation 50 degrees, right lateral rotation 50 degrees with tightness and pain in the bilateral paracervical muscles, right greater than left. Bilateral paracervical moderate myofascial hypertrophy and tenderness, left greater than right.

THORACIC SPINE: Bilateral rhomboids mild to moderate myofascial hypertrophy and tenderness, left greater than right.

LUMBAR SPINE: Forward flexion 70 degrees, extension 20 degrees with tightness and pain noted in the lumbosacral muscles. Bilateral lumbosacral myofascial trigger point spasming, hypertrophy, tenderness to palpation.

EXTREMITIES: Left shoulder with decreased range of motion, pain on flexion/extension, mild Neers and Hawkins, tenderness to palpation over the posterior glenohumeral region.

Right shoulder with full range of motion, negative Neers/Hawkins, mild tightness reported in the right paracervical muscles on end shoulder flexion, no tenderness to palpation.

NEURO: Awake, alert, and oriented x 3. Negative Romberg w/ no sway with eyes closed. Full sensation in upper and lower extremities to pinprick and light touch. +5/5 strength in upper and lower extremities bilaterally, 2/4 deep tendon reflexes bilaterally. No gait ataxia with tandem heel-toe walk. No finger-to-nose dysmetria. Negative Spurling's bilaterally. Negative straight leg raises bilaterally.

DIAGNOSTICS

Cervical spine MRI 12/13/2024 at Axos Imaging - Multilevel annular ruptures/tears associated with disc protrusion measuring up to 4.9 mm, resulting in severe left neural foraminal narrowing and mild-to-moderate spinal canal stenosis and mild bilateral neural foraminal narrowing.

Lumbar spine MRI 12/13/2024 at Axos Imaging - Multilevel annular ruptures/tears associated with facet arthrosis and bilateral disc protrusion measuring up to 5 mm, resulting in mild spinal canal stenosis and moderate bilateral neural foraminal narrowing.

Head MRI with TBI protocol on 1/5/2025 at Axos Imaging - 10 foci of increased T2/FLAIR signal intensity within the bilateral frontal parietal centrum semiovale and corona radiata white matter. Increased volume of the cortical gray matter in the 99th percentile compared to age and gender-matched controls. DTI data indicate 2 out of 48 regions demonstrate decreased fractional

anisotropy ($p < 0.05$), affecting regions involved in transmitting proprioceptive and sensory information, and memory and emotional processing.

TRIGGER POINT INJECTION PROCEDURE NOTE

Informed consent discussed regarding risks/benefits of Trigger Point Injections including potential for bleeding, bruising and/or infection. Patient verbalized full understanding of risks/benefits and agreed to proceed with TPIs at clinic bedside. Injection sites cleansed with alcohol swabs and **Trigger Point Injections x 2 performed to bilateral paracervical muscles** using 25 g 1" needle with 1 cc mixture of 3 cc Bupivacaine 0.5% and 0.5 cc Kenalog 40 mg/ml injected into each trigger point site with good immediate relief and no complications noted. Additional **Trigger Point Injections x 2 performed to bilateral lumbosacral muscles** using 25 g 1" needle with 1 cc mixture of 3 cc Bupivacaine 0.5% and 0.5 cc Kenalog 40 mg/ml injected into each trigger point site with good immediate relief and no complications noted.

Each injection site/trigger point massaged with focal acupressure for 1 minute and sites bandaged with no post procedural complications noted. Prior to completion of appointment, patient reported moderate relief of his trigger point discomfort with good anesthetic effect at each injection site.

Home care instructions advised including keeping injection sites clean&dry, avoiding applying direct heat to injection sites and leaving bandages on for 12 hours. Call back parameters discussed.

ASSESSMENT

This is a 36-year-old male, who was injured in a motor vehicle collision on 11/13/2024, sustaining injuries to his neck, mid/low back and bilateral shoulders. He also reports persistent mild/moderate headaches, dizziness, blurry vision, brain fog, and decreased memory, attention and concentration, anxiety and insomnia since his MVC.

V89.2XXA Person injured in motor vehicle traffic accident

S06.0 Concussion

S13.4xxA Sprain of ligament of cervical spine

S16.1xxA Strain of muscle, fascia and tendon at neck level

M50.90 Cervical disc disorder

S23.3xxA Sprain of ligament of thoracic spine

S29.012A Strain of muscle and tendon of back wall of thorax

S33.5XXA Sprain of ligaments of lumbar spine

S39.012A Strain of muscle, fascia and tendon of lower back

M51.86 Intervertebral disc displacement, lumbar region

M25.519 Pain in unspecified shoulder
S43.42 Sprain of rotator cuff capsule
S46.00 Injury of muscles and tendons of the rotator cuff
F43.22 Adjustment disorder with anxiety
F43.0 Acute stress reaction
G47.9 Sleep Disorder, unspecified
G47.01 Insomnia due to medical condition

TREATMENT PLAN

1. Headaches/Dizziness/Impaired Cognition – Timeline of symptoms, positive Romberg, gait ataxia, and cognitive impairment are consistent with diffuse Traumatic Brain Injury/post concussion symptoms. Cognitive/physical rest, avoidance of EtOH advised. NSAIDs prn, Omega-3 FA, Super B complex, Turmeric capsules, Magnesium supplementation, and stress reduction techniques advised. **Cognitive assessment scheduled for completion to measure the extent of cognitive impairment to further formulate a cognitive rehab plan. For post-concussive headaches, recommending occipital nerve block injection for improved relief.**
2. Neck pain - **Continue current chiropractic therapy sessions with Dr. Zach Beatty.** Continue current prescribed analgesics regimen. Heating pads and over-the-counter lidocaine cream/diclofenac path as needed for relief. **Trigger Point Injections x 2 to the bilateral paracervical muscles given today** with good relief and no complications noted. May consider performing repeat trigger point injections at the next visit for additional relief. Should symptoms persist/worsen, may consider referral for a potential Epidural Steroid Injection (ESI) to offer improved relief.
3. Mid back/lower back pain - Chiropractic care and pain management as outlined above. **Trigger Point Injections x 2 to the bilateral lumbosacral muscles given today** with good relief and no complications noted. May consider performing repeat trigger point injections at the next visit for additional relief. Should symptoms persist/worsen, may consider referral for a potential ESI to offer improved relief.
4. Bilateral shoulder pain – Consistent with RTC tendinitis/impingement syndrome, possible RTC tear and labral tear. **Left shoulder MRI recommended to further evaluate.** Chiropractic care and pain management as outlined above. **Activity modifications advised, not to lift anything more than 30 pounds.** May consider performing Platelet Rich Plasma (PRP) vs. Cortisone Injection depending on MRI results and for improved relief should symptoms persist/worsen.
5. Anxiety/Adjustment Disorder – Deep breathing/meditation, progressive muscle relaxation, guided imagery and desensitization/re-exposure techniques discussed. Naturopathic supplementation with L-Theanine, GABA or Ashwagandha advised if still

symptomatic. Should the above measures fail, will consider prescribing psychotropic anxiolytic vs antidepressant medications for improved relief and referring to Psychologist for structured Cognitive Behavioral Therapy.

6. Insomnia – Proper sleep hygiene measures discussed including setting consistent sleep schedule, shortening sleep time if laying awake in bed for sustained periods of time to strengthen association between time in bed with time asleep (sleep consolidation technique), minimize screen time/stress prior to bedtime and gentle stretching, light aerobic activity each day. Naturopathic supplementation as outlined above.

We will schedule the patient back into the clinic to be seen on **4/03/2025 at 11:20 AM in our Mission Valley office** or earlier as needed for reevaluation.

DISCLAIMER/DISCLOSURE NOTE

I declare under the penalty of perjury that the name and qualification of each person who performed any service in connection with this report including diagnostic study and clerical preparation are as follows: Amber Cargill, PA-C for history, physical examination, comprehensive evaluation and generation of this report to a meaningful form.

I declare under penalty of perjury that I personally performed the evaluation and physical examination of the patient or, in the case of a supplemental report, I personally performed the cognitive services necessary to produce the report and that, except as otherwise stated herein, the evaluation performed and the time spent performing the evaluation was in compliance with the guidelines.

By my signature on the report and this affidavit, I certify that this report represents the work product of myself and my staff in the manner described and expresses exclusively my professional opinion, findings, and conclusions on the matters discussed in the report.

Sincerely,

Handwritten signature of Amber Cargill, PA-C in cursive script.

Amber Cargill, PA-C
NPI: 1477267755
CA License: PA63926

Case discussed with PA Cargill, report reviewed and I am in agreement with assessment/plan as outlined above.

Sincerely,

A handwritten signature in black ink, appearing to read "Paul A. Jain, M.D.", with a stylized flourish at the end.

PAUL A. JAIN, M.D., Q.M.E.

San Diego/San Marcos/Chula Vista/Murrieta/La Mesa/Santee

Diplomate, American Board of Internal Medicine

Diplomate, National Board of Physicians and Surgeons

Qualified Medical Examiner

PAUL A. JAIN, M.D., Q.M.E.

3838 Camino Del Rio N, STE 240

San Diego, CA 92108

(619) 880-8844

Other locations: San Marcos, Chula Vista, Murrieta, La Mesa, Santee

*Diplomate, American Board of
Internal Medicine*

*Diplomate, National Board of
Physicians and Surgeons*

Qualified Medical Examiner

July 31, 2025

RE: Peikert, Thomas
D/B: 03/9/1988
D/I: 11/13/2024
D/E: 07/31/2025

FOLLOW UP INTERNAL MEDICINE REPORT

Mr. Peikert presents with ongoing good improvement in his left shoulder pain after 3 platelet rich plasma injections, which he describes as approximately 70% better than before the treatment. He has had minimal flare ups of pain in the past 3 weeks. When he does experience pain, it is primarily when performing overhead movements or fast actions and is a sharp, burning, non radiating pain. Thomas is currently on light duty at work, which has helped manage his symptoms.

Thomas also notes mild neck and back pain described as muscle aches, but otherwise has been improving. He also experiences daily headaches, described as a pressure sensation in the occipital region, lasting 30 minutes to an hour, and relieved by rest. Thomas reports intermittent blurry vision, which has improved over time, and associates it with physical activity at work.

For pain management, the patient takes the prescribed diclofenac and tizanidine as needed, approximately once a week. He recently completed chiropractic treatment and is not currently engaged in formal therapy.

HISTORY OF PRESENT ILLNESS

Please refer to the initial report for full HPI.

MEDICAL HISTORY

CHRONIC MEDICAL
CONDITIONS:

Sleep Apnea
Hypertension (currently not on any medication)

SURGERIES:

Tonsillectomy during young childhood

ALLERGIES:

NKDA

No other medical conditions reported, will review should new medical records be available.

CURRENT MEDICATIONS

Diclofenac 75 mg q12h prn
Lidocaine 4% ointment q12h prn
Tizanidine 2 mg q12h prn

PHYSICAL EXAMINATION

HEIGHT: 5'11"

WEIGHT: 230 pounds

BLOOD PRESSURE: **138/90 mmHg, left arm sitting**

PULSE: 69 beats per minute and regular

RESPIRATIONS: 16 breaths per minute

PULSE OXIMETRY: 97% on room air

GENERAL: The patient appeared to be in **mild distress**. He ambulated without the use of an assistive device but with no difficulty noted while sitting on the examination table.

HEENT: Normocephalic and atraumatic. PERRLA, EOMI b. No nystagmus noted b. b TMJs non tender to palpation with jaw opening/closing, no audible/palpable click noted. **Mild dizziness blurry vision with extraocular movements and horizontal saccades**, but able to

complete all 10 trials. Visual Acuity test: **Left eye 20/25. Right 20/40. Both 20/30.**

CERVICAL SPINE: Cervical flexion 40 degrees, extension 30 degrees. Left lateral rotation 50 degrees with tightness and mild pain reported in the left paracervical muscles on end range of motion, right lateral rotation 50 degrees with tightness and mild pain reported in the midline cervical spine on end range of motion. Bilateral paracervical mild to moderate myofascial hypertrophy and tenderness, left greater than right.

THORACIC SPINE: Bilateral rhomboids mild myofascial hypertrophy and tenderness, left greater than right.

LUMBAR SPINE: Forward flexion 40 degrees, extension 20 degrees with tightness and pain reported in the lumbosacral muscles. Bilateral lumbosacral myofascial mild hypertrophy, tenderness to palpation.

EXTREMITIES: Left shoulder with decreased range of motion, pain reported in the left paracervical muscles on end shoulder flexion/extension, positive Neers and Hawkins, tenderness to palpation over the supraspinatus, infraspinatus, and subacromial bursa.

Right shoulder with full range of motion, negative Neers/Hawkins, mild tightness reported in the right paracervical muscles on end shoulder flexion, no tenderness to palpation.

NEURO: Awake, alert, and oriented x 3. Negative Romberg w/ no sway with eyes closed. Full sensation in upper and lower extremities to pinprick and light touch. +5/5 strength in upper and lower extremities bilaterally, 2/4 deep tendon reflexes bilaterally. No gait ataxia with tandem heel-toe walk. No finger-to-nose dysmetria. Negative Spurling's bilaterally. Negative straight leg raises bilaterally.

DIAGNOSTICS

Cervical spine MRI 12/13/2024 at Axos Imaging - Multilevel annular ruptures/tears associated with disc protrusion measuring up to 4.9 mm, resulting in severe left neural foraminal narrowing and mild-to-moderate spinal canal stenosis and mild bilateral neural foraminal narrowing.

Lumbar spine MRI 12/13/2024 at Axos Imaging - Multilevel annular ruptures/tears associated with facet arthrosis and bilateral disc protrusion measuring up to 5 mm, resulting in mild spinal canal stenosis and moderate bilateral neural foraminal narrowing.

Head MRI with TBI protocol on 1/5/2025 at Axos Imaging - 10 foci of increased T2/FLAIR signal intensity within the bilateral frontal parietal centrum semiovale and corona radiata white matter. Increased volume of the cortical gray matter in the 99th percentile compared to age and gender-matched controls. DTI data indicate 2 out of 48 regions demonstrate decreased fractional anisotropy ($p < 0.05$), affecting regions involved in transmitting proprioceptive and sensory information, and memory and emotional processing.

Left shoulder MRI 3/20/2025 at Axos Imaging - Partial undersurface tear of the distal supraspinatus tendon. Fraying of the distal infraspinatus and distal subscapularis tendons. Tendinosis of the long head of the biceps in the rotator interval. Grade 1 acromioclavicular injury. Tears of the anterior/superior and posterior/superior labrum. Mild flattening and cortical irregularity of the anterior/medial humeral head with underlying subcortical edema representing non-displaced reverse humeral Hill-Sachs impaction fracture, sequelae of posterior shoulder subluxation/dislocation.

TRIGGER POINT INJECTION PROCEDURE NOTE

Informed consent discussed regarding risks/benefits of Trigger Point Injections including potential for bleeding, bruising and/or infection. Patient verbalized full understanding of risks/benefits and agreed to proceed with TPIs at clinic bedside. Injection sites cleansed with alcohol swabs and **Trigger Point Injections x 2 performed to bilateral paracervical muscles** using 25 g 1" needle with 1 cc mixture of 3 cc Bupivacaine 0.5% and 0.5 cc Kenalog 40 mg/ml injected into each trigger point site with good immediate relief and no complications noted.

Each injection site/trigger point massaged with focal acupressure for 1 minute and sites bandaged with no post procedural complications noted. Prior to completion of appointment, patient reported moderate relief of his trigger point discomfort with good anesthetic effect at each injection site. Home care instructions advised including keeping injection sites clean&dry, avoiding applying direct heat to injection sites and leaving bandages on for 12 hours. Call back parameters discussed.

ASSESSMENT

This is a 37-year-old male, who was injured in a motor vehicle collision on 11/13/2024, sustaining injuries to his neck, mid/low back and bilateral shoulders. He also reports persistent mild/moderate headaches, dizziness, blurry vision, brain fog, and decreased memory, attention and concentration, anxiety and insomnia since his MVC.

V89.2XXA Person injured in motor vehicle traffic accident
S06.0 Concussion
F07.81 Postconcussional Syndrome
S06.2X0A Diffuse traumatic brain injury
G31.84 Mild cognitive impairment
S13.4xxA Sprain of ligament of cervical spine
S16.1xxA Strain of muscle, fascia and tendon at neck level
M50.90 Cervical disc disorder
S23.3xxA Sprain of ligament of thoracic spine
S29.012A Strain of muscle and tendon of back wall of thorax
S33.5XXA Sprain of ligaments of lumbar spine
S39.012A Strain of muscle, fascia and tendon of lower back
M51.86 Intervertebral disc displacement, lumbar region
S43.42 Sprain of rotator cuff capsule
S46.00 Injury of muscles and tendons of the rotator cuff
M25.512 Pain in left shoulder
M75.42 Impingement syndrome of left shoulder
S43.432D Superior glenoid labrum lesion of left shoulder, subsequent encounter
S42.29 Other fracture of upper end of humerus
S43.52X Sprain of left acromioclavicular joint
S46.012A Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder,
F43.22 Adjustment disorder with anxiety
F43.0 Acute stress reaction
G47.9 Sleep Disorder, unspecified
G47.01 Insomnia due to medical condition

TREATMENT PLAN

1. Headaches/Dizziness/Impaired Cognition – Timeline of symptoms, positive Romberg, gait ataxia, and cognitive impairment are consistent with diffuse Traumatic Brain Injury/post concussion symptoms. **Cognitive/physical rest, enough sleep, avoidance of EtOH, and stress reduction advised. Omega-3 FA/Fish Oil, Vitamin B12, Turmeric, and Magnesium supplementation are recommended for better brain health.** Should post-concussive headaches persist/worsen, may consider performing occipital nerve block injection for improved relief.
2. Neck pain - Status post completion of chiropractic therapy sessions with Dr. Zach Beatty. Continue current prescribed analgesics regimen. Heating pads and over-the-counter lidocaine cream/diclofenac patch as needed for relief. **Trigger Point Injections x 2 to the bilateral paracervical muscles given today with good relief and no complications noted.** May consider performing repeat TPis at the next visit for additional relief.

3. Mid back/lower back pain - Chiropractic care and pain management as outlined above. May consider performing repeat trigger point injections at the next visit for additional relief. Should symptoms persist/worsen, may consider referral for a potential ESI to offer improved relief.
4. Bilateral shoulder pain, left greater than right – Status post left shoulder PRP injections x 3 given on 5/02/2025, 5/30/2025, and 6/27/2025 with good relief and continues improvement per patient report. **Activity modifications advised, not to lift anything more than 30 pounds. Home exercises for range of motion and strengthening discussed.** Should symptoms persist/worsen, may consider submitting a referral to an Orthopedic Specialist for surgical recommendations.
5. Anxiety/Adjustment Disorder – Deep breathing/meditation and naturopathic supplements as previously discussed. Should the above measures fail, will consider prescribing psychotropic anxiolytic vs antidepressant medications for improved relief.
6. Insomnia – Proper sleep hygiene measures as previously discussed. Naturopathic supplementation with Ashwagandha is recommended if still symptomatic. If using Calm Magnesium Sleep Formulation, take caution not to combine w/ additional Melatonin, GABA, or L-Theanine.

DISCLAIMER/DISCLOSURE NOTE

I declare under the penalty of perjury that the name and qualification of each person who performed any service in connection with this report including diagnostic study and clerical preparation are as follows: Rachel Kennedy, PA-C for history, physical examination, comprehensive evaluation and generation of this report to a meaningful form.

I declare under penalty of perjury that I personally performed the evaluation and physical examination of the patient or, in the case of a supplemental report, I personally performed the cognitive services necessary to produce the report and that, except as otherwise stated herein, the evaluation performed and the time spent performing the evaluation was in compliance with the guidelines.

By my signature on the report and this affidavit, I certify that this report represents the work product of myself and my staff in the manner described and expresses exclusively my professional opinion, findings, and conclusions on the matters discussed in the report.

Sincerely,



Rachel Kennedy, PA-C

NPI: 1871370577

CA License: PA66344

Case discussed with PA-C Kennedy, report reviewed and I am in agreement with assessment/plan as outlined above. Given the patient's ongoing symptomatology, future medical care as outlined below is required.

FUTURE MEDICAL CARE

Future medical care includes an in depth Neuropsychiatric assessment at a cost of \$8,000 and Mr. Peikert should have a full series of Cognitive Rehab sessions for his Traumatic Brain Injury/post concussion syndrome, up to 24 sessions per year at a cost of \$250-300/session (\$7,200).

For persistent headaches, future medical care includes neurofeedback therapy, up to 2x/weekly or 50 sessions per year at a cost of \$150-200/session (\$10,000). In addition,

For his neck and low back pain, future medical care includes more Chiropractic Therapy or a trial of Physical Therapy care, up to 24 sessions per year at a cost of \$175-250/session (\$6,000). In addition, Mr. Peikert will need a trial of Acupuncture with therapeutic massage up to 24 sessions per year at a cost of \$150-200/session (\$4,800).

For neck and low back pain, should his pain symptoms persist/recur despite exhaustive conservative management, Mr. Peikert will require a referral to an Interventional Pain Management Specialist (\$1,000) for evaluation and consideration of potential injections including Trigger Point Injections (\$500 each for paracervical and lumbosacral regions), Facet (\$2000 each for cervical/lumbar spine) and/or a series of 3 cervical and lumbar epidural steroid injections at a cost of \$6,000/injection (\$36,000).

For his left shoulder pain, should his pain symptoms persist despite exhaustive conservative management and after completing 3 PRP injections, Mr. Peikert will require a referral to an Orthopedic Extremity Specialist (\$1,000) for evaluation and for likely an arthroscopic shoulder surgery at a global cost of \$40,000.

Total estimated future medical care cost is \$119,000.

We will schedule the patient to be seen back in the clinic as needed for reevaluation.

Sincerely,

A handwritten signature in black ink, appearing to read "Paul A. Jain, M.D.", with a stylized flourish at the end.

PAUL A. JAIN, M.D., Q.M.E.

San Diego/San Marcos/Chula Vista/Murrieta/La Mesa/Santee

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